

BLOCK 2
**HEALTH CARE: PLANNING,
POLICY AND MANAGEMENT**

BLOCK 2 HEALTH CARE: PLANNING, POLICY AND MANAGEMENT

Introduction

Unit 6 Health Care: Historical Perspective: Describe the Alma Ata Declaration and the turnaround of events in health sector. It also explains the term primary health care and right to health in terms of its postulates. This also critically analyse some of the major developments in the history of health care and explain its significance in today's world.

Unit 7 Health Policy in India: Development of health policy is elaborated. Health policy and planning in India in different phases has been evaluated. Delivery of various National Health Programmes under planning are critically evaluated. The challenges in India's health policy are looked in terms of accessibility, affordability is also looked into.

Unit 8 Role of Technology: Health Statistics, GIS and Health Information Systems: Understand the existing role of technology in shaping rural health is explained in this unit. Health Information Systems, Hospital Information System, Electronic Health Records, telemedicine, e-health which are technology driven measures health care are dealt. In this unit the challenges and scope of health-related technologies, especially for rural India are also identified.

Unit 9 Health Care: Governmental and Non-Governmental Initiatives: Explain the role of government organizations in providing holistic healthcare to its population. It also describes non-governmental healthcare initiatives in India. It critically analyse how government and non-government organizations can work together for 'Health For All'.

Unit 10 Health Infrastructure and Delivery System: India's health system in terms of state and central health services; public and private and voluntary sectors; allopathic and non-allopathic doctors, qualified and unqualified practitioners and multiple levels has been explained in this unit. Health financing and manpower are also included in infrastructure of health delivery systems.

In the end in this block, you will be able to understand the interface of technology in health care. Technology made people constantly search for information about their health on the internet now they have already got knowledge of the disease before consulting a doctor. Pharmaceuticals are selling their products online, hospitals are buying their products online, and billing insurance companies using the internet, therefore the need for a drastic shift from the normal way of conducting health affairs. The utilization of information technology can't be ignored within hospitals.

UNIT 6 HEALTH CARE: HISTORICAL PERSPECTIVE

Structure

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6.0 INTRODUCTION

Health is a positive multi-dimensional concept involving a variety of features, ranging from ability to integrity, from fitness to well-being. Although science has provided a foundation for healthcare, social values have shaped the system. The history of healthcare is replete with pioneers whose discoveries led to strong opposition and sometimes violent rejection by conservative elements and vested interests in medical, public or political circles. The boundaries of healthcare have changed over time with the perception of new health and social problems and with political, economic and ideological shifts within the government and nations

6.1 LEARNING OUTCOMES

After reading this Unit, you should be able to:

- explain the concept of health and the evolution and significance of holistic healthcare;
- describe the Alma Ata Declaration and the turnaround of events in its rise and fall;
- explain the term 'primary health care' and some of its postulates or understanding;

- explain the term ‘right to health’; and
- critically analyse some of the major developments in the history of health care and explain its significance in today’s world.

6.2 EVOLUTION OF THE CONCEPT OF HEALTH

The history of health has been one of identifying health problems, developing knowledge and expertise to solve problems with rallying political and social support around the solutions. According to the first principle of the constitution of the World Health Organization “Health is a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity”. This constitution was adopted by the International Health Conference held in New York in 1946 and became operative in April 1948. This classical, seventy-year-old definition of the World Health Organization is now considered a historical one and stands as a fundamental milestone.

The 1960s and 1970s was an era of newly won independence from former colonial powers for many countries. This independence was accompanied by an enthusiasm to provide high-standard healthcare, education and other services for the people. Governments moved to establish teaching hospitals and medical and nursing schools, often with the assistance of donor nations. These tertiary services consumed the largest portion of the country’s healthcare budget and were available mostly in urban areas, creating access problems for the predominantly rural societies. Healthcare services to the rural majority were supplied by missionary hospitals and clinics or by touring services provided from urban hospitals. There was a wide variety of services of varying standard and quality in the rural areas with most of the population visiting traditional healers. By the 1970s, the morbidity and mortality for rural communities was not improving, and in some places they deteriorated. Developments such as oral rehydration solutions, showed that early and appropriate intervention by carers and village volunteers could avoid referral and admission to hospital and if combined with an effectively organised vaccination program, would address the major causes of death and illness. China, Tanzania, Sudan and Venezuela initiated successful programs to deliver a basic but comprehensive program of primary care health services covering poor rural populations. This new methodology for healthcare service delivery incorporated a questioning of top-down approaches and the role of the medical profession in healthcare provision and was known as ‘primary health care’.

During the 1970s, World Health Organization and UNICEF together addressed the need for a fundamental change in the delivery of healthcare services in developing countries, with an emphasis on equity and access at affordable cost and emphasising prevention while still providing appropriate curative services. It was after that it was understood that primary health care reflects and evolves from the economic, sociocultural and political conditions of the country and its communities and is based on relevant application of social, biomedical and health services research. Primary healthcare requires and promotes maximum community and individual self-reliance and participation in its planning, organization, operation and control, making fullest use of local and national

resources. It relies, at local and referral levels, on health workers, including physicians, nurses, midwives, auxiliaries and community workers, traditional practitioners and technical health teams to respond to community needs. It should be sustained by integrated, functional and mutually supportive referral systems, leading to progressive improvement of comprehensive health care with priority to those most in need.

Check Your Progress: 1

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) Explain the concept of health as it is understood in the contemporary times.

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2) How the concept of health has evolved over the years?

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6.3 DECLARATION OF ALMA ATA

While there had been a recognition of the importance of health as a reflection of social determinants, this recognition was formalised into policy in 1978 with 134 national government members of the World Health Organisation. The Declaration made Primary Health Care the official health policy of all member countries with health as a human right based on the principles of equity and community participation. Alma Ata broadened the perception of health beyond doctors and hospitals to social determinants and social justice. It is a history that started with a consensus of the United Nations agencies supporting a view of health as a human right and as a result of social determinants to a narrower view defining universal health coverage as a focal point of policy implementation. The Alma Ata Declaration stated that governments have a responsibility for the health of their people which can be fulfilled only by the provision of adequate social measures and that the people have the right and duty to participate individually and collectively in the planning and implementation of their health care.

The Alma-Ata Declaration defined health as a state of complete physical, mental and social well-being and not mere absence of diseases. This definition of health along with the concepts of primary health care formed the two core principles of this declaration. It recognised health as a human right – and to attain the same, several social and economic sectors in addition to the health sector need to be actively involved and show results.

The Declaration further affirmed the crucial role of primary health care, which addresses the main health problems in the community, providing promotive,

preventive, curative and rehabilitative services accordingly. It stresses that access to primary health care is the key to attaining a level of health that will permit all individuals to lead a socially and economically productive life and to contributing to the realization of the highest attainable standard of health through the following key postulates:

- 1) Health is a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity. It is a fundamental human right and the attainment of the highest possible level of health is the most important world-wide social goal whose realization requires the action of many other social and economic sectors in addition to the health sector.
- 2) The existing gross inequality in the health status of the people particularly between developed and developing countries as well as within countries is politically, socially and economically unacceptable and is, therefore, of common concern to all countries.
- 3) Economic and social development, based on a New International Economic Order, is of basic importance to the fullest attainment of health for all and to the reduction of the gap between the health status of the developing and developed countries. The promotion and protection of the health of people is essential to sustained economic and social development and contributes to a better quality of life and world peace.
- 4) People have the right and duty to participate individually and collectively in the planning and implementation of their health care.
- 5) Governments have a responsibility for the health of their people which can be fulfilled only by the provision of adequate health and social measures.
- 6) Primary health care is essential health care based on practical, scientifically sound, socially acceptable and technologically accessible to communities wrapped in the spirit of self-reliance and self-determination. It forms an integral part of the country's health system and the overall social and economic development of the community. It is the first level of contact of individuals with health care brought as close as possible to where people live and work.
- 7) All governments should formulate national policies, strategies and plans of action to launch and sustain primary health care as part of a comprehensive national health system.
- 8) All countries should cooperate in a spirit of partnership and service to ensure primary health care for all people since the attainment of health by people in any one country directly concerns and benefits every other country.

A multi-sectorial approach was a strong pillar of Alma Ata Declaration. It declared that unequal social and economic status within the country itself as well as among the countries contribute to ill health and pronounced the urgent need to break or at least reduce these huge gaps. Perhaps, the most important declaration was that it held the governments responsible for taking care of the health of its citizens. It envisaged universal coverage of basic services such as education on methods of preventing and controlling prevailing health problems,

promotion of food security and proper nutrition, adequate safe water supply and basic sanitation, maternal and child health, family planning, vaccination, prevention and control of locally endemic diseases, appropriate treatment of common diseases and provision of essential drugs. The emphasis changed from the larger hospital to that of community-based delivery of services with a balance of cost-effective preventive and curative programmes. The approach has been intersectoral, involving agriculture extension officers, school teachers, women's groups, youth groups and the community itself in the implementation of its own healthcare services after being trained as a formal part of the healthcare system.

The conference went on to address the economic and political steps needed to fund the initiative of an acceptable level of health for all people of the world by the year 2000. It believed to attain the same through a fuller and better use of the world's resources and a genuine policy of independence, peace and disarmament towards the acceleration of social and economic development of which primary health care, as an essential part, should be allotted its proper share. National governments throughout the world adopted PHC as their official blueprint for total population coverage and goals and targets were set for achieving 'Health for All' by the Year 2000. In the initial stages, nurses and health extension officers were trained in clinical and preventive PHC interventions to work in community health centres. Where there were gaps in the healthcare system, village health workers were with community representatives having a central role in planning and overseeing their healthcare services.

6.3.1 The Rise and Fall of Alma Ata Declaration

The Alma-Ata Declaration was endorsed by the 32nd World Health Assembly held in Geneva in 1979, which approved a resolution acknowledging the key role of primary health care for the attainment of acceptable health levels for all. However, despite the initial enthusiasm, it was difficult to implement healthcare in accordance with Alma-Ata principles. Early efforts at expanding primary health care in the late 1970s and early 1980s were overtaken in many parts of the developing world by economic crisis, sharp reductions in public spending, political instability and emerging disease. The social and political goals of Alma Ata provoked early ideological opposition and were never fully embraced in market oriented, capitalist countries.

The efforts to dismantle Alma-Ata Declaration brick by brick began in the year 1979 when the Rockefeller Foundation sponsored a conference in Italy, wherein it was argued that Alma-Ata's Primary Health Care concept was expensive and unrealistic. The cornerstone of Alma-Ata, which was community participation, was struck down, and instead, a selective, politically sanitised version of PHC reduced to a few highly prioritised technological interventions, determined not by communities but by international health agents and experts was strongly advocated. This came under the new label as Selective Primary Health Care. The governments that largely catered to the interests of the rich were quick enough to grab this as they had vested interests in the form of preserving status quo of the unequal economic and social conditions. It was in response to this that in the year 1983, UNICEF adapted 'GOBI-FFF' (Growth monitoring, ORS, Breastfeeding, Immunisation, Family planning, Food supplements and Female education). Many governments just selected ORS and immunisation, which

the UNICEF proclaimed as ‘twin engines’ of child survival revolution. Several corporate-friendly countries, USAID and World Bank which had shown scant respect for the Alma-Ata Declaration, suddenly jumped onto the bandwagon and pledged major financial support. By mid 1980s, virtually every underdeveloped countries had launched GOBI-FFF intervention.

Politicians and aid experts from developed countries could not accept the core PHC principle that communities in developing countries would have responsibility for planning and implementing their own healthcare services. The concept of ‘Selective Primary Health Care’ advocated providing only PHC interventions that contributed most to reducing child (< 5 years) mortality in developing countries. The advocates of SPHC argued that comprehensive PHC was too idealistic, expensive and unachievable in its goals of achieving total population coverage. By focusing on growth monitoring, oral rehydration solutions, breastfeeding and immunisation, greater gains in reducing infant mortality rates could be achieved at reduced cost. In effect, SPHC took the decision-making power and control central to PHC away from the communities and delivered it to foreign consultants with technical expertise in these specific areas. These technical experts, often employed by the funding agencies, were subject to the policies of their agencies, not the communities. SPHC reintroduced vertical programmes at the cost of comprehensive PHC.

The PHC versus SPHC debate continued throughout the 1980s with the following arguments among others:

- Many ordinary people felt PHC was a cheap form of healthcare and bypassed this level to attend secondary and tertiary centres because of a lack of staff and essential medicines at the PHC level.
- Civil war, natural disasters and HIV affected the ability of PHC to maintain comprehensive services, especially in many sub-Saharan countries.
- Political commitment was not sustained after the initial euphoria of Alma-Ata. Politicians saw PHC as a way to reduce expenditure in health and lacked the political will to ensure that services were equitable.
- Issues of governance and corruption in the use of resources resulted in donors becoming very wary of funding comprehensive, broad-based programs. Vertical, definable, time-limited programmes that could be changed every few years suited both donor agencies and governments.

By 1990, many developing countries had started initiating privatisation and liberalisation processes under the duress of World Bank and IMF. But when ‘Health For All by 2000’ was not achieved, several health activists from all over the world sat down at Savar, in Dhaka leading to the People’s Health Movement, with its presence in 70 countries campaigning for Health Rights, through People’s Health Charter as a hope to revive the Alma-Ata Declaration.

Check Your Progress: 2

- Note:**
- 1) Use the space provided below for your answers.
 - 2) Compare your answers with those given at the end of the unit.

- 1) What was the Declaration of Alma Ata? State two of its postulates.

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- 2) Why did the Alma Ata Declaration fail?

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6.3.2 Health Sector Reform: The World Bank Report, 1993

Changes in political and economic philosophy in the late 1980s and 1990s marked a major change in how government services were delivered throughout the world. The fall of the socialist eastern European bloc and China's adoption of many aspects of liberal economics were major features of this period. These reforms had their roots in placing the emphasis on reducing government involvement in all aspects of society. Market forces became the dominant model for service delivery. Governments in resource-poor countries, which had already reduced their expenditure on health as their foreign debt mounted, had to contend with the new economic philosophy. International donors insisted these governments adopt the market-driven economic reforms if they were to receive foreign aid and debt relief.

It was against this background that the World Bank's World Development Report of 1993, 'Investing in Health', was undertaken. It reflected a marked change in the orientation of how healthcare services in resource-poor countries would be delivered with little reference to the term Primary Health Care. It considered the delivery of healthcare services in terms of the economic benefit that improved health could deliver. The report revolves around healthcare sector activities in improving health with scant recognition to the role of other sectors, in stark contrast with the original PHC's multisectoral approach. This World Bank approach came to be known as Health Sector Reform. This heralded an emphasis on using the private sector to deliver healthcare services while reducing or removing government services. User pays, cost recovery, private health insurance and public-private partnerships became the focus for delivery of healthcare services. This did not give communities in developing countries a say directly or indirectly in their health services with a sense of inequity, marginalisation and frustration.

6.4 RIGHT TO HEALTH

The Right to Health is not new and is a fundamental part of the human rights. Internationally, it was first articulated in the 1946 Constitution of the World Health Organization as 'the enjoyment of the highest attainable standard of health is one of the fundamental rights of every human being without distinction of race, religion, political belief, economic or social condition'.

Article 25.1 of the Universal Declaration of Human Rights deems that “Everyone has the right to a standard of living adequate for the health of himself and of his family, including food, clothing, housing and medical care and necessary social services.”

Right to Health is fundamentally important in order to have a transparent and quality health care service in the country and address health crisis as well as scenarios like medical terrorism. Additionally, it bestows financial protection for a vast majority of health care needs, bring down their out-of-pocket expenditure on health care, keeping the sanity of right to freedom of healthcare intact.

The right to health is an inclusive right that includes safe drinking water and adequate sanitation; safe food; adequate nutrition and housing; healthy working and environmental conditions; health-related education and information and gender equality. The right to health also mandates freedom including the right to be free from non-consensual medical treatment such as medical experiments and research or forced sterilization and to be free from torture and other cruel, inhuman or degrading treatment or punishment. Further, the right secures non-discriminatory health services, goods and facilities to the highest attainable standard of health. Though not surprisingly, traditionally discriminated and marginalized groups often bear a disproportionate share of health problems.

The right to health is also recognized in several regional instruments, such as the African Charter on Human and Peoples’ Rights (1981), the Protocol of San Salvador (1988) and the European Social Charter (1961, revised in 1996). The American Convention on Human Rights (1969) and the European Convention for the Promotion of Human Rights and Fundamental Freedoms (1950) also contain provisions related to health, such as the right to life, the prohibition on torture and other cruel, inhuman and degrading treatment and the right to family and private life. The right to health or health care is recognized in at least 115 constitutions with six other constitutions that set out duties, such as the duty on the State to develop health services or to allocate a specific budget to them.

The 1948 Universal Declaration of Human Rights mentioned health as part of the right to an adequate standard of living in its Article 25. The right to health was again recognized as a human right in the 1966 International Covenant on Economic, Social and Cultural Rights. Since then, other international human rights treaties have also recognized or referred to the right to health or to elements of it (such as the right to medical care). The right to health is relevant to all states and every state has ratified at least one international human rights treaty recognizing the right to health. Moreover, states have committed themselves to protecting this right through international declarations, domestic legislation and policies and at international conferences. In recent years, increasing attention has been paid to the right to the highest attainable standard of health, for instance by human rights treaty- monitoring bodies, WHO and the Human Rights Council to clarify the nature of the right to health and how it can be achieved.

The Right to Health contains the following entitlements:

- The right to a system of health protection providing equality of opportunity for everyone to enjoy the highest attainable level of health.

- The right to prevention, treatment and control of diseases.
- Access to essential medicines.
- Maternal, child and reproductive health.
- Equal and timely access to basic health services.
- The provision of health-related education and information.
- Participation of the population in health-related decision making at the national and community levels

The term Right to health is mentioned as a fundamental right under Right to life enshrined in Article 21 of the Indian Constitution. There are multiple references in the Constitution to public health and on the role of the State in the provision of healthcare to citizens:

- Article 39 lays down principles of policy to be followed by the State:

that the health and strength of workers, men and women, and the tender age of children are not abused and that citizens are not forced by economic necessity to enter avocations unsuited to their age or strength;

that children are given opportunities and facilities to develop in a healthy manner and in conditions of freedom and dignity and that childhood and youth are protected against exploitation and against moral and material abandonment.

- Article 42 directs the State to just and humane conditions of work and maternity relief.
- Article 47 informs the State of its duty to raise the level of nutrition and the standard of living and to improve public health.

The State regards the raising of the level of nutrition and standard of living of its people, improvement of public health, bring about prohibition of the consumption except for medicinal purpose of intoxicating drinks and drugs which are injurious to health. Moreover, the Constitution does not only oblige the State to enhance public health, it also endows the Panchayats and Municipalities to strengthen public health under Article 243G .

Human rights are interdependent, indivisible and interrelated. Violating the right to health may actually impair the enjoyment of other human rights too. The importance given to the underlying determinants of health beyond health services, goods and facilities, shows that the right to health is dependent on and contributes to the realization of other human rights. These include the rights to food, to water, to an adequate standard of living, to adequate housing, to freedom from discrimination, to privacy, to access to information, to participation and the right to benefit from scientific progress and its applications.

Check Your Progress: 3

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) Explain the right to health. Highlight it in the context of Indian Constitution.

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6.5 SUSTAINABLE DEVELOPMENT GOALS

The United Nations Millennium Declaration was signed in September 2000 towards committing world leaders to combat poverty, hunger, disease, illiteracy, environmental degradation and discrimination against women. The Millennium Development Goals were derived from this Declaration as a set of 8 goals that all 189 UN Member States agreed to try to achieve by the year 2015. Each MDG had targets set for 2015 and indicators to monitor progress from 1990 levels. At the United Nations Millennium Summit in September 2001, heads of 147 states endorsed the MDGs, nearly half of which concern different aspects of health. India's progress towards health related MDGs was mixed. The nation achieved the required trend reversal in the fight against HIV/AIDS.

While on the worldwide platform, deliberations regarding MDGs were going on as it was realised that they created by only a few stakeholders without adequate involvement by developing countries and overlooking development objectives previously agreed upon. It was understood that MDGs were unachievable and simplistic, not adapted to national needs, do not specify accountable parties and reinforce vertical interventions. The MDGs have been superseded by the Sustainable Development Goals, a set of 17 integrated and indivisible goals that build on the achievements of the MDGs but are broader, deeper and far more ambitious in scope, including new, interdependent challenges such as climate change, economic inequality, sustainable consumption, peace and justice, among others. Contrary to the MDGs, this inclusive agenda launched in September 2015 by the UN General Assembly, applies to all countries, rich and poor, although each country is expected to push forward the different targets in accordance with its own priorities.

Health has a central position in the agenda through SDG 3, and is closely linked to over a dozen targets in other goals related to urban health, equal access to treatments, and non-communicable diseases, among others. SDGs represent a unique opportunity to promote public health through an integrated approach to public policies across different sectors. While only SDG 3, to ensure healthy lives and promote well-being for all at all ages, focuses on human health, all goals are interrelated. For example, better education for girls (goal 4.1) would improve maternal health (goal 3.1); tackling child malnourishment (goal 2.2) would have a great impact on child health (goal 3.2); and ensuring access to safe water (6.1) or tackling ambient air pollution (11.6) will evidently have a direct impact on several SDG3 targets. On the other hand, using coal to improve energy access (goal 7), would have a negative impact on health. The achievement of the health goals will need policy coherence to reinforce synergies between certain SDGs and minimise trade-offs.

Check Your Progress: 4

Health Care: Historical Perspective

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) Explain the significance and objective of good health in terms of the Sustainable Development Goals.

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6.6 NATIONAL HEALTH POLICY, INDIA

The Ministry of Health and Family Welfare, Government of India, evolved a National Health Policy in 1983 based on the preventive, promotive, public health and rehabilitation aspects of healthcare. The policy stressed the need for establishing comprehensive primary health care services to reach the population in the remote areas of the country and recommended a decentralized system of health care, the key features of which were low cost and participative with an aim to reduce the government's burden.

Going ahead, a revised health policy for achieving better health care and unmet goals was brought out by government of India as the National Health Policy 2002 with the following objectives and key Strategies:

- Primary Health Care Approach.
- Decentralized public health system.
- Convergence of all health programme under single field umbrella.
- Strengthening and extending public health services.
- Enhanced contribution of private and NGO sector in health care delivery.
- Increase in public spending for health care.

Further in 2017, the Indian government revised the National Health Policy to attain the highest possible level of health and well-being for all at all ages through a preventive and promotive health care orientation in all developmental policies, in line with the SDG-3 with support from the United Nations.

It is commendable that NHP-2017 recognizes the SDG's to be of pivotal importance, identifying seven priority areas outside the health sector which can have an impact on preventing and promoting health, those being air pollution, better solid waste management, water quality, occupational safety, road safety, housing, vector control, and reduction of violence and urban stress.

With regard to the improvement of maternal and child health, the policy seeks to address the social determinants through developmental action in all sectors. It states that research on social determinants of health will be promoted, combining this with neglected health issues such as disability and transgender health; while giving importance on Panchayati raj institutions to play an

enhanced role at different levels for health governance, including the social determinants of health.

In highlighting the need for an empowered public health cadre, the NHP explains the need to address social determinants of health effectively, by enforcing regulatory provisions. The insertion of gender based violence in national programmes, and the call for increased sensitization of health systems to provide care free and with dignity in the public and private sector, are innovative contributions to better health care. NHP is patient centric and quality driven policy that addresses health security and Make-In-India for drugs and devices. The policy proposes raising public health expenditure to 2.5 per cent of the GDP in a time bound manner. It advocates extensive deployment of digital tools for improving the efficiency and outcome of the healthcare system. It looks at problems and solutions holistically with private sector as strategic partners. It seeks to promote quality of care focusing on emerging diseases and investment in promotive and preventive healthcare. It envisages private sector collaboration for strategic purchasing, capacity building, skill development programmes, awareness generation, development of sustainable networks for community to strengthen mental health services, and disaster management. The policy also advocates financial and non-incentives for encouraging private sector participation.

The primary aim of the NHP 2017 is to inform, clarify, strengthen and prioritize the role of the Government in shaping health systems in all its dimensions- investment in health, organization and financing of healthcare services, prevention of diseases and promotion of good health through cross sectoral action, access to technologies, developing human resources, encouraging medical pluralism, building the knowledge base required for better health, financial protection strategies and regulation and progressive assurance for health. With NHP 2017 and related regulatory frameworks there is a sense of optimism that India will make Sustainable progress towards attaining “Health for All” and the SDG targets.

Check Your Progress: 5

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) Explain the evolution of the National Health Policy of India over the years.

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6.7 INTERNATIONAL HEALTH REGULATIONS (2005)

The International Health Regulations (IHR), first adopted by the World Health Assembly in 1969 are a legally binding instrument of international law that aims for international collaboration to prevent, protect against, control and provide a public health response to the international spread of disease in ways

that are commensurate with and restricted to public health risks and that avoid unnecessary interference with international traffic and trade.

While the original International Health Regulations were founded in 1969, but its underpinnings can be traced to the mid-19th century, when measures to tackle the spread of plague, yellow fever, smallpox and cholera across borders, with as little interference to global trade and commerce, were debated. To address the realisation that countries varied with regards to their sanitary regulations and quarantine measures, the first of these series of early international sanitary conferences, convened in Paris in 1851. In 1948, the World Health Organization Constitution came about and in 1951, issued their first infectious disease prevention regulations, the 'International Sanitary Regulations', which focussed on six quarantinable diseases; cholera, plague, relapsing fever, smallpox, typhoid and yellow fever. These were revised and renamed the 'International Health Regulations' in 1969.

In 2005, following the 2002–2004 SARS outbreak, changes were made to overcome the following challenges against the backdrop of the increased travel and trade characteristic of the 20th century:

- narrow scope of notifiable diseases with the emergence of new infectious agents (Ebola Haemorrhagic Fever in Zaire and the re-emergence of cholera and plague in South America and India, respectively).
- lack of a formal internationally coordinated mechanism to prevent the international spread of disease.

With the signing of the revised International Health Regulations (IHR) in 2005, the international community agreed to improve the detection and reporting of potential public health emergencies worldwide. The 2005 IHR came into force in June 2007, with 196 binding countries that recognised that certain public health incidents, extending beyond disease, ought to be designated as a Public Health Emergency of International Concern, as they pose a significant global threat. Its first full application was in response to the swine flu pandemic of 2009.

IHR better addresses today's global health security concerns and are a critical part of protecting global health. The regulations require that all countries have the ability to detect, assess, report and respond to public health events.

- **Surveillance:** Surveillance systems can detect acute public health events in timely matter.
- **Assess and report:** Assess public health event and report to WHO through their National IHR Focal Point.
- **Respond:** Respond to public health risks and emergencies.

While disease outbreaks and other acute public health risks are often unpredictable and require a range of responses, the International Health Regulations provide an overarching legal framework that defines countries' rights and obligations in handling public health events and emergencies that have the potential to cross borders. The International Health Regulations represent an agreement between 196 countries who have agreed to build their capacities to detect, assess and

report public health events. WHO plays the coordinating role in IHR and, together with its partners, helps countries to build capacities.

International Health Regulations also includes specific measures at ports, airports and ground crossings to limit the spread of health risks to neighbouring countries and to prevent unwarranted travel and trade restrictions so that traffic and trade disruption is kept to a minimum. While previous regulations required countries to report incidents of cholera, plague, and yellow fever, IHR (2005) is more flexible and future-oriented, requiring countries to consider the possible impact of all hazards, whether they occur naturally, accidentally or intentionally.

In spite of broader global agreement to the importance of IHR (2005), only about 1/3 of the countries in the world currently have the ability to assess, detect, and respond to public health emergencies, leaving them vulnerable.

Check Your Progress: 6

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) How do the International Health Regulations help its member countries to fight public health responses?

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6.8 LET US SUM UP

The last few decades have seen the move of a consensus on a global health policy across a wide range of experiences in successes and failures of policy implementation. In today's interconnected society, it's more important than ever to make sure all countries realise and understand the concept of health care to be able to respond to and contain public health threats:

There is no 'blue print' for universal implementation of PHC policy. PHC is not a biomedical intervention that is linear and generalisable. Unlike programmes to eradicate specific diseases focusing on the need for specific personal behaviours such as accepting immunisations or accessing clean water, PHC addresses a range of approaches that support a change of both opinions and behaviours that only happen over a long period of time and reflect a specific context amenable to these changes. It depends on the ability of governments to raise funds, on the availability of funds for the health sector and competing interests for their allocation. It also depends on the availability of human resources to support health programmes and on the structures to allocate and apply these resources to specific programmes. Further, it depends on the organisation and management of health care delivery in both government and non-government programmes.

National governments that have established Primary Health Care as a policy focus and implemented this policy do not have the same programmes. For example, Thailand has pursued Primary Health Care focused on Universal

Health Coverage. India and Ethiopia have focused on Community Health based Workers (CHWs) programmes while Brazil created Family Health Teams composed of one doctor, one nurse, one nurse assistant and between four to six CHWs. Countries choose and develop programmes that best suit their context and needs. Despite obstacles and limitations, these programmes have been sustainable and have been noted as successes in getting health care to poor populations over a long time period.

PHC policy implementation is a process that develops over time and with experience. The process of primary health care planning and implementation involves trial and error, learning from mistakes and responding with flexibility to meet problems and needs. Seeing the implementation of PHC as a process highlights the dynamics of the application of the intervention. It also highlights the factors that encourage or limit its acceptance or rejection. It demands a recognition and investigation into both intended and unintended outcomes. It demands a continual monitoring of what works, why does it work and how it works. It also demands a continual interaction between policy makers, programme managers, health providers, intended beneficiaries and other stakeholders such as non-government organisations and individuals and communities. When programmes make achievements, health outcomes are seen in sustainable health programmes particularly at the community level that have been able to address issues around equity and community acceptance, involvement and support for health improvements and health care delivery.

‘Health for All’ will remain a dream as long as it is formulated in purely technocratic terms – drugs, nurses, doctors, vaccines and X-ray equipment. One needs to look beyond the four walls of the hospitals where the real causes of ill-health lurk. It could be unemployment, underemployment, poor housing with overcrowding, lack of access to nutritious food and safe water, illiteracy and gender inequalities. To overcome all this, people will need to be involved in planning health programmes. And for that, a minimum understanding of health is essential. So, in essence, social awareness and health go hand in hand. Identifying this being an important component is known as community involvement.

6.9 KEY WORDS

Health: Health is a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity.

Healthcare: Health care is the maintenance or improvement of health via prevention, diagnosis, treatment, recovery or cure of disease, illness, injury or physical and mental impairments in people.

Primary Health Care: The healthcare service delivery model that reflects and evolves from economic, sociocultural and political conditions of the country and its communities with maximum community and individual participation is called Primary Health care.

Declaration of Alma Ata: The Alma Ata Declaration was the recognition of the importance of health as a reflection of social determinants, formalised into

policy in 1978 with 134 member countries of the World Health Organisation. It made Primary Health Care the official health policy of all these countries with health as a human right based on the principles of equity and community participation.

Right to Health: The Right to Health the enjoyment of the highest attainable standard of health is one of the fundamental rights of every human being without distinction of race, religion, political belief, economic or social condition. It is a fundamental part of the human rights.

Millennium Development Goals: The United Nations Millennium Development Goals (MDGs) are 8 goals that UN Member States have agreed to try to achieve by the year 2015.

Sustainable Development Goals: The Sustainable Development Goals (SDGs) were adopted by all United Nations Member States in 2015 as a universal call to action to end poverty, protect the planet and ensure that all people enjoy peace and prosperity by 2030.

International Health Regulations: The International Health Regulations (IHR), first adopted by the World Health Assembly in 1969 are a legally binding instrument of international law that aims for international collaboration to prevent, protect against, control and provide a public health response to the international spread of disease in ways that are commensurate with and restricted to public health risks and that avoid unnecessary interference with international traffic and trade.

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6.11 CHECK YOUR PROGRESS: POSSIBLE ANSWERS

Check Your Progress: 1

1. Refer to Section 6.2

Check Your Progress: 2

1. Refer to Section 6.3
2. Refer to Section 6.3.1

Check Your Progress: 3

1. Refer to Section 6.4 (pg 98)

Check Your Progress: 4

1. Refer to Section 6.5 (pg 100)

Check Your Progress: 5

1. Refer to Section 6.6

Check Your Progress: 6

1. Refer to Section 6.7

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UNIT 7 HEALTH POLICY IN INDIA

Structure

- 7.0 Introduction
- 7.1 Learning Outcomes
- 7.2 Meaning of Health Policy
- 7.3 Health Policy and Planning in India Just After Independence
- 7.4 Health for All by the Year 2000 AD
- 7.5 Health Policy -1983: An Appraisal
- 7.6 National Health Policy-2002
- 7.7 National Rural Health Mission (NRHM) -2005
- 7.8 National Health Policy-2017
- 7.9 Ayushman Bharat Yojna
- 7.10 Delivery of National Public Health Programmes
- 7.11 Challenges and Management of Health Care
- 7.12 Let Us Sum Up
- 7.13 Key Words
- 7.14 Further Readings
- 7.15 Check Your Progress: Possible Answers

7.0 INTRODUCTION

Health care planning has undergone changes in its structure, personnel and in its access of the populace on the basis of recommendations made by different committees. After independence, India has initiated several policies and programmes and global partnership has been introduced, so far expected results has been not shown, therefore without the realization of the goals of the programme of action of the millennium development which combines clear theoretical thinking with realistic application. Though multi-sectoral approach has been adopted at various stages, but still there is a need to be percolated these aspects in the grass-root through trained and well skilled workers for sustainable impact of health and family welfare programmes. In reality vast infrastructure in health care facilities is able to cater only twenty per cent of the population and eighty per cent of the people's health needs are met by private health care provision. Restricted access to health facilities leads to higher incidence of morbidity and mortality. Further, it also forces people to have out-of-pocket expenses which often lead to indebtedness. In rural areas and poor people in urban areas are victim of the vicious circle of poverty, malnutrition and poor health reinforcing each other.

7.1 LEARNING OUTCOMES

After studying this Unit, You will be able to:

- understand the meaning of health policy;
- discuss health policy planning in India over a period of time;
- delivery of National Health Programmes; and
- find out the challenges in health policy

7.2 MEANING OF HEALTH POLICY

Health policy can be defined as the decisions, plans, and actions that are undertaken to achieve specific healthcare goals within a society. According to WHO, an explicit health policy can achieve several things: it defines a vision for the future; it outlines priorities and the expected roles of different groups; and builds consensus and informs people. The policy process encompasses decisions made at a national or state level that affect whether and how services are delivered. Policy is in way national law that support different programmes related to it and make intervention accordingly. When health policy is made, it is followed by the operational policies. Operational policies are the rules, regulations, guidelines, and administrative norms that governments use to translate policies into the field situation whether they are to be delivered.

Health policy can be defined as the decisions, plans, and actions that are undertaken to achieve specific healthcare goals within a society.

Check Your Progress: 1

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1. What do you mean by health policy?

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7.3 HEALTH POLICY AND PLANNING IN INDIA JUST AFTER INDEPENDENCE

With the advent of British Rule, every facet of Indian life– including the medical and public health services– was subordinated to commercial, political and administrative interests of the imperial (state) government in the United Kingdom and its representatives in India. However, along with their own, the Britishers also permitted native Indian elite also to avail of modern health care. Keeping in view the constitutional directions aiming at elimination of poverty and ill health, the Government of India, soon after Independence, planned several approaches for health-care delivery. The basis of the health development policy in India was laid in the recommendations of the Health Survey and Development Committees, namely, Bhore Committee as early as

in 1946, Mudaliar Committee (1961), Chadah Committee (1963), Mukherjee Committee (1965), Jungalwala Committee (1967), Jain Committee (1968), Kartar Singh Committee (1973), and Srivastava Committee (1975). Besides the Committees, inputs have been provided by some studies on health policy, which were conducted by the autonomous institutions like the ICSSR and the ICMR. The important recommendations by these committees can be thematically grouped under the following: (a) for improving the existing institutional structure of health care delivery system, (b) to improve the supporting services for a proper and quick delivery of medi-care facilities, and (c) to improve the quality of the delivery system thereby improve the health standard of the masses (Nagla: 1993). Health care planning has undergone changes in its structure, personnel and in its access of the populace on the basis of recommendations made by different committees. Nagla (1993) has highlighted the enormous efforts made by the various committees to chart out the growth of health manpower in India. The works of these committees have, however, been subject to serious data limitations. Information on different categories of health personnel, their priorities and distribution (especially in respect to professionals outside the public sector) has been rather scanty. The various committees and even institutions like the ICSSR/ICMR study group have been focusing on providing expert opinions rather than garnering accurate statistical data. No committee has been able to take into account the contribution of non-governmental health institutions and that of professionals in rendering health care services. Perhaps more importantly, the committees have been unable to assess the health care needs of the people through field surveys and ascertain the extent of utilization of health care institutions and practitioners (Yesudin: 1981). Introduction of economic reforms and liberalization has created space for social insurance schemes introduced by public sector Life Insurance Corporation and by private insurance companies, introducing medical insurance policies. This was the initiation of public-private mix for health-care support in India.

7.4 HEALTH FOR ALL BY THE YEAR 2000 AD

In order to provide minimum basic health facilities, it was resolved by the Health Assembly of the WHO to launch a movement known as “Health for All by the Year 2000 AD”. In 1978, the Alma-Ata Conference reaffirmed Health for All as the major social goal of all governments. In 1981, a global strategy for this programme was adopted by the WHO, which was later endorsed by the United Nations General Assembly. India is a signatory to those declarations and has made efforts to extend health facilities to the vulnerable sections of its society.

Health for All has been defined as attainment of “a level of health that will enable every individual to lead a socially and economically productive life” (Government of India: 1982). The concept of “Health for All by 2000 AD” implied a substantial change in basic health policies and in the approaches to health care. In order to provide minimum health facilities, the government of India along with state governments increased the allocation to the health sector. A group set up jointly by the ICSSR and ICMR with an alternative strategy to achieve health for all citizens. It suggested certain steps for restructuring

the health care service infrastructure based on the principle of promoting the preventive and curative aspects of health. This group felt that, “the growth of health care services in the country has been haphazard and unrelated to the needs of poor and rural people who stand most in need of health care”. The group remarked that several assumptions on which present system is based were wrong. For instance, there was no distinction between planning for health and that for health services so that little or no attention has been paid to the social, economic, political and cultural dimensions of health. The recommendations of the study group were more practical and provided details of health services up to the village level. It emphasized that most common illnesses can be self-cured and need only symptomatic treatment with simple remedies, be it herbal, indigenous or allopathic medicine. The diseases, which are communicable, can be controlled by preventive measures, and can be readily diagnosed and treated with the help of cheap and highly effective drugs. The group, therefore, proposed that primary health services should be extended to the community through a trained and involved community health volunteer preferably male or female, for a population of 1,000. To lift the status of preventive and promotive aspects of medical services, the working group has suggested that greater emphasis be given to departments of preventive and social medicine.

7.5 HEALTH POLICY -1983: AN APPRAISAL

National Health Policy-1983, framed in a spirit of optimistic empathy for the health needs of the people, particularly the poor and under-privileged, had hoped to provide ‘Health for All by the Year 2000 AD’, through the universal provision of comprehensive primary health care services. The genesis of health sector reforms and its various dimensions are in built in Indian policy instrument and its developmental framework. However, several pertinent issues in health sector reforms have a bearing on our constitutional provisions—like equity, accessibility of poor, efficiency in using resources and inadequacy of resources, gender bias, quality of health care, impact of liberalization etc. It has been observed after analyzing the health policy that the financial resources and public health administrative capacity is inadequate and therefore the set goals for the policy could not be achieved. The main recommendation of NHP is to increase the broad-based availability of health services to the masses on the basis of realistic considerations of capacity.

7.6 NATIONAL HEALTH POLICY-2002

The main objective of the NHP-2002 is to achieve an acceptable standard of good health amongst the people especially the poor people. To make health services available to the people, the decentralization of public health system is targeted. It can be done by establishing new infrastructure in deficient areas and by upgrading the infrastructure in the existing institutions. Importance was given to the equitable access to the health services across all sections of society and particularly the poor people and people living in remote areas. It was also emphasized that aggregate public health spending will be increased by the central government. In turn, this initiative can strengthen the capacity of the public health administration at the State level to render effective service

delivery. In NHP, importance was given to the preventive and first-line curative initiatives at the primary level through increased sectoral share of allocation. The rational use of drugs within the allopathic system is also preferred. Increasing use of generic drugs over the branded drugs is also pushed on particularly in the government health facilities. Access to tried and tested systems of traditional medicine was also ensured.

The total expenditure in health expenditure is around 5.2 per cent of Gross Domestic Product (GDP). However, the between 2012 and 2013 aggregate health spending is expected to reach a rate of 5.5 per cent by 2013. Looking to these figures it is concluded that the expenditure on public health services is very meagre and below from the desirable standard. However, under the Constitutional structure, public health is the responsibility of the States. Under the federal structure it is the states that provide the health services to their population and central government helps them in certain National programmes and national Health initiatives. In NHP-2002 it was proposed that the central government will spend 6 per cent of GDP, with 2 per cent of GDP being contributed as public health investment by the year 2010. However, it is the failure of the government that it has not done what it envisaged in the NHP-2002 (National Health Policy: 2002). To reduce these inequalities and imbalances at inter regional level, rural- urban level, between different economic classes, caste groups, the primary health sector is targeted. Primary health sector has been given importance over tertiary and secondary health sector. The NHP- 2002 also projected that the increased aggregate outlays for primary health sector will boost the existing facilities and also opens the door for new primary health centres and sub-centres according to the norms of such facilities. The policy for the eradication and elimination of disease requires a major role of the central government with the active participation of the State governments. In the public health facilities, the outdoor funding is insufficient. The medical and para-medical personnel are also much less of the prescribed norms. The availability of consumable items is negligible. The equipment in many public health facilities is outdated or obsolescent and unusable. Buildings are in the dilapidated. Some facilities are in the rented accommodations. In the indoor facilities, there is insufficiency of beds, equipment are obsolescent. Availability of essential drugs is minimal and the capacity of facilities is grossly inadequate. Over-crowding is another feature in the public health facilities which results into the steep deterioration of quality of health services delivered to the people. This results into the decreasing use of public health facilities i.e. around 20 per cent. Only less than 20 per cent seek out door patient service and less than 45 per cent avail indoor facilities in the public health facilities. This is the situation in spite of peoples' inability to pay for the private services and results into the out-of-pocket payments. In principle, this Policy welcomes the participation of the private sector in all areas of health activities – primary, secondary or tertiary. The policy is encouraging the private insurance companies for increasing the coverage of secondary and tertiary sector under health insurance packages. In the National Health Framework, the alternative systems of medicine like Ayurveda, Yoga, Unani, Siddha and Homeopathy (AYUSH) are also put under one umbrella to have their substantial role in the health care.

7.7 NATIONAL RURAL HEALTH MISSION (NRHM)-2005

National Rural Health Mission was launched in April 2005 in to provide health care to the rural people particularly to the vulnerable section, children and women. The basic aim is to provide comprehensive and integrated health care to the rural masses. In this mission, there is a provision to provide every village with trained female community health activist, known as Accredited Social Health Activist (ASHA). She works as mediator between the community and the public health system. These ASHA workers would not get any remuneration, rather they would be provided only performance-incentives for promoting universal immunization, referral and escort services for reproductive and child health and other programmes. The Central government has framed this Rural Health Care Mission at policy level, but it is the responsibilities of the State governments to executed and support it.

7.8 NATIONAL HEALTH POLICY: 2017

The basic aim of National Health Policy- 2017 is to improve health status through concerted policy action in all sectors and expand preventive, promotive, curative, palliative and rehabilitative services provided through the public health sector with focus on quality. The policy envisages as its goal the attainment of the highest possible level of health and well-being for all at all ages, through a preventive and promotive health care orientation in all developmental policies, and universal access to good quality health care services without anyone having to face financial hardship as a consequence. This would be achieved through increasing access, improving quality and lowering the cost of healthcare delivery. The policy gives importance to the Sustainable Development Goals (SDGs). The policy aims to draw upon the diverse systems of medicine practiced in India and the different sectors of healthcare provider. The policy acknowledges the need for raising the level of public financing for health and determines to raise it to 2.5 per cent of the GDP by 2025. It affirms that 70 per cent of funding should go to primary healthcare, which is needed for comprehensive care and promotion of well being. Health and wellness Centres are established which connects the family on the basis of family cards. Health and Wellness Centre provides basic health services, referral linkages and perform a gatekeeper function for advanced care. More advanced care is opened for secondary and tertiary level medical centres. 'Fee for service' model is adopted for secondary and tertiary care. 'Capitation mode' of payment will be there for primary care. Expanded institutional capacity, and new courses and cadres are proposed to overcome the shortages of skilled human resources in the health system. Essential drugs and diagnostic are provided free of cost at public facilities.

7.9 AYUSHMAN BHARAT YOJNA

Ayushman Bharat (Long Life to India) was launched in 2018 and renamed as Pradhan Mantri Jan Arogya Yojna (PMJAY). It proposes to convert 1,50,000 primary health sub-centres into Health and Wellness Centres (HWC). These

centres will be redesigned to provide a wide range of essential primary health services close to home and will have both facility-based and community outreach services. It will become a platform for health services related to maternal and child health, communicable and non-communicable diseases, mental health and common disorders. It is proposed that each HWC will serve 5000 persons. A Primary health Centre (PHC) will have six HWCs connected to it. Along with wellness promotion through integration of other systems of medicine, promotion of healthy diets, regular physical activity, yoga, advocacy of tobacco avoidance and substance abuse, vector control, sanitation, hygiene and clean water programmes will be promoted.

Check Your Progress: 2

Note: 1) Use the space provided below for your answers.
2) Compare your answers with those given at the end of the unit.

1) Summarize the health policy just after independence.

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2) What was the basic goal of Health for All?

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3) What was the goal of National Health Policy of 1983?

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4) What is the main objective of National Health Policy of 2002?

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5) What is National Rural Health Mission?

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6) Give the critical point of National Health Policy of 2017.

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7.10 DELIVERY OF NATIONAL PUBLIC HEALTH PROGRAMMES

It is self-evident that in a country as large as India, which has a wide variety of socio-economic settings, national health programmes have to be designed with enough flexibility to permit the State public health administrations to craft their own programme package according to their needs. Also, the implementation of the national health programme can only be carried out through the State Governments' decentralized public health machinery.

Over the last decade or so, the Government has relied upon a 'vertical' implementational structure for the major disease control programmes. Through this, the system has been able to make a substantial dent in reducing the burden of specific diseases. However, such an organizational structure, which requires independent manpower for each disease programme, is extremely expensive and difficult to sustain. Over a long time-range, 'vertical' structures may only be affordable for those diseases, which offer a reasonable possibility of elimination or eradication in a foreseeable time-span.

It is a widespread perception that, over the last decade and a half, the rural health staff has become a vertical structure exclusively for the implementation of family welfare activities. As a result, for those public health programmes where there is no separate vertical structure, there is no identifiable service delivery system at all. The Policy will address this distortion in the public health system.

This policy envisages a key role for the Central Government in designing national programmes with the active participation of the State Governments. Also, the Policy ensures the provisioning of financial resources, in addition to technical support, monitoring and evaluation at the national level by the Centre. However, to optimize the utilization of the public health infrastructure at the primary level, NHP-2002 envisages the gradual convergence of all health programmes under a single field administration. Vertical programmes for control of major diseases like TB, Malaria, HIV/AIDS, as also the RCH and Universal Immunization Programmes, would need to be continued till moderate levels of prevalence are reached. The integration of the programmes will bring about a desirable optimization of outcomes through a convergence of all public health inputs. The Policy also envisages that programme implementation be effected through autonomous bodies at State and district levels. The interventions of State Health Departments may be limited to the overall monitoring of the achievement of programme targets and other technical aspects.

The Policy envisages that apart from the exclusive staff in a vertical structure for the disease control programmes, all rural health staff should be available for the entire gamut of public health activities at the decentralized level, irrespective of whether these activities relate to national programmes or other public health initiatives.

7.11 CHALLENGES AND MANAGEMENT OF HEALTH CARE

The provision of health under Indian constitution is fall in State subject. Today every state is free to formulate their health programme and strategy. In fact, India was the first country in the world, who introduce family planning programme in 1952. Since Independence, we have invested lot in the health infrastructure and created 1,45,000 sub centres, 2,3000 Public Health Centres (PHCs) and 3222 Community Health Centres (CHCs). However, this infrastructure is still insufficient for the vast population, as still approximately eighty per cent of the population is receiving health care from the private sector. Poor, access to public health facilities heads to higher incidence of morbidity and mortality.

The poor people have an out-of-pocket expense which often leads to indebtedness. The vicious circle of indebtedness leads to poverty, malnutrition and poor health. They reinforce each other. In India, private households' contribution to healthcare is 75 per cent. Most of these costs are out-of-pocket costs. Private Expenditure on Health (PHE) as a per cent of per capita income has almost doubled since 1961. Private Expenditure on Health has increased at much higher rate than the per capita income over the period of time.

The 'vertical' or; categorical' programmes were not properly conceived, too techno-centric programmes, which are imposed by the western countries over the poor developing countries without understanding of their specificity. International Monetary Fund (IMF), while giving the loans has also imposed certain conditionalities in the form of imposition of international initiative in early phase of 1990s. The Structural Adjustment Programme (SAP) enabled the entry into most important elements of the governance of the country in the form of influencing budgetary allocations in the country. SAP has also given break through to the market of private health sector rapidly. There is a sharp decline in the government's commitment to public health services which is reflected in various plan outlays. India's public Health expenditure in terms of its percentage from the GDP, it is the fifth lowest in the World. Private sector has been provided many subsidies and rebate in the import of medical instruments and for drugs. India has the largest and least regulated private healthcare industry in the world.

The increasing role of the state in delivering health care services and the consequent bureaucratization of health care delivery has created a divergence between governmental goals and targets for collective health status and the individual's efforts to maintain health and well-being. While the lack of governmental health care is found to adversely affect the health status of a population, greater interference of medical services through the government bureaucracy and the rapid diffusion of privatised medical care lead to inequality of health status.

Universalization of health care is at the heart of health policy in India since Independence in 1947. Since then, the country has experienced different models to improve access to health services and ensure universal care to entire population. Despite the proliferation of governmental and private initiatives, the majority of the Indian population still has great difficulty in obtaining

appropriate health services. Along problems of spatial distribution of health services, other barriers stand between patients and health care services.

Weaknesses in the public sector are noted across the entire chain of facilities, from sub-centres to medical college hospitals. This has been due to inadequate investment in infrastructure, human resources and equipment, compounded by poor management.

The private sector has flourished due to the void created by the dwindling capacity and competence of the public sector and aided by mismanagement and weak regulatory system. The large corporate hospitals are charging high cost of care and insensitive to human sufferings due to crass commercial considerations.

The models of public-private partnership (PPP) deployed so far have not worked well because of poorly designed contracts, vaguely defined deliverables and inadequately enforced accountability. The dependence on the private sector for service delivery in government-funded health insurance schemes has yielded valuable lessons on how unregulated partnership can lead to unintended consequences of induced demand and unnecessary procedures.

The success or failure of the policy is mainly judged in its equity component. In the framework of equity dimension, a marked emphasis has been given to the expansion and improvement for the primary health facilities including the new concept of provisioning of essential drugs through central funding. The improvement in the health status of the people depends upon the quality of health services. Increase in financial and material input is not only sufficient for improvement in the health status but it also depends on more empathetic and committed attitude in the service providers in the private and public sectors. Ultimately it is the quality of the health services which matters for the enhanced health status. In this backdrop, it needs to be recognized that any policy in the social sector is critically dependent on the service providers treating their responsibility not as a commercial activity, but as a service, albeit a paid one. In the public health sector, an enhanced standard of governance is a prerequisite for the success of any health policy.

Check Your Progress: 3

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) How vertical programmes are used for the disease control?

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2) Mentions the challenges encountered in health care.

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7.12 LET US SUM UP

This unit briefly summarizes the health policy and planning in India since independence. Health care planning has undergone changes in its structure, personnel and in its access of the populace on the basis of recommendations made by different committees. After independence, India has initiated several policies and programmes and global partnership has been introduced, so far expected results has been not shown, therefore without the realization of the goals of the programme of action of the millennium development which combines clear theoretical thinking with realistic application. Though multi-sectoral approach has been adopted at various stages, but still there is a need to be percolated these aspects in the grass-root through trained and well skilled workers for sustainable impact of health and family welfare programmes. The infrastructure in health still insufficient for the vast population, as still approximately eighty per cent of the population is receiving health care from the private sector. Poor, access to public health facilities heads to higher incidence of morbidity and mortality. The poor people have an out-of-pocket expense which often leads to indebtedness. The vicious circle of indebtedness leads to poverty, malnutrition and poor health. They reinforce each other. In India, private households' contribution to healthcare is 75 per cent. Most of these costs are out-of- pocket costs.

7.13 KEY WORDS

Health Policy: Health policy is a vision document for the future; it outlines priorities and the expected roles of different groups; and builds consensus and informs people.

Health for All: It has been defined as attainment of a level of health that will enable every individual to lead a socially and economically productive life.

Out-of-Pocket Cost: Individual's inability to pay for the private health services results in out-of-pocket cost.

Fee-for-Service: It means providing payment for each hospitalization which includes medical consultation, procedure or test performed.

Capitation Model: It is a fixed payment for a whole set of health services covering a person or a family in year or any other defined period to cover healthcare costs incurred for a person in a year.

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7.15 CHECK YOUR PROGRESS: POSSIBLE ANSWERS

Check Your Progress: 1

1. Refer to Section 7.2

Check Your Progress: 2

1. Refer to Section 7.3
2. Refer to Section 7.4
3. Refer to Section 7.5
4. Refer to Section 7.6
5. Refer to Section 7.7
6. Refer to Section 7.8

Check Your Progress: 3

1. Refer to Section 7.11
2. Refer to Section 7.11

UNIT 8 ROLE OF TECHNOLOGY: HEALTH STATISTICS, GIS AND HEALTH INFORMATION SYSTEMS

Structure

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8.0 INTRODUCTION

Most of the developing countries are facing a significant problem within the sector of health care. Technology made people constantly search for information about their health on the internet now they have already got knowledge of the disease before consulting a doctor. This change has not only affected the patients but also other stakeholders too. Pharmaceuticals are selling their products online, hospitals are buying their products online, and billing insurance companies using the internet, therefore the need for a drastic shift from the normal way of conducting health affairs. Hence the utilization of information technology can't be ignored within hospitals.

The healthcare sector represents an enormous opportunity to leverage technology to enhance critical processes that today pose a huge challenge within the delivery of quality healthcare. This includes reaching millions who are geographically spread

across the country, providing a far better and more accurate diagnosis, managing operations, and facilitating effective collaboration and dialogue between doctors and healthcare workers. The health care condition is very poor and it is a challenging issue mostly in providing health care services to people living in rural and remote areas. The application of technology has brought tremendous signs of progress in the health care sector. Especially the Information Communication and Technology (ICT) have a growing influence on all areas of human life. In an effort to cope up with the growth, the developing countries have endeavoured to transform the healthcare industry by use of Information Technology in different ways. The technologies can improve the quality, safety, and efficiency of health services and delivery. The health information units within the nation should get provided and actively engaging other sectors such as education, planning, statistics, finance, ICT to accelerate reliable health information. Health information is a national asset, balancing affordability, accessibility and efficiency are important to universal health care systems.

Approximately 70 per cent of India's population residing in rural areas. Rural and urban areas differ in several ways, including demography, environment, economy, social construction, and availability of resources. There are significant health disparities and access to care issues that are specific to rural areas. The state of local communities in rural areas is often considered inadequate since the standard of medical care provided in rural areas has generally been appeared to be substandard to that of the urban settings. The reason being that the rural inhabitants are in general more likely than urban inhabitants to have the lower educational achievement, experience high unemployment, live in poverty, and additionally are more likely to be ineligible for welfare benefits. Additionally, when it involves accessing to health care, the rural population has been viewed as vulnerable with reference to access to healthcare due to poorly established and fragile health infrastructures, the high prevalence rate for chronic illness and disability, socio-economic hardships, and physical barriers like distance including lack of public transportation.

Growing up in India, where the mainstream has minimal access to basic public amenities like healthcare, one develops an appreciation for the significance of enabling such technologies that assist and scale service delivery within the context of developing countries. Technology is often transformative in delivering healthcare services, where the density of doctors is one per 1,000. A strong imbalance exists where urban areas have fourfold as many medical practitioners compared to rural areas, making rural India grossly underserved. The quality of education of providers (allopathic, ayurvedic, unani, and homeopathic) isn't similar between rural and urban areas. When 58 per cent of the doctors in urban areas had a medical degree, only 19 per cent of these in rural areas had that qualification. The country is 81 per cent in necessity of specialists at rural Community Health Centers (CHCs), and therefore the private sector interpretations for 63 per cent of hospital beds, reliable with government health and family welfare statistics.

E-health are often termed as any electronic exchange of health-related data through electronic connectivity for enlightening competence and effectiveness of health care delivery. The solutions that are provided through e-health initiatives within hospitals include Hospital Information Systems (HIS), telemedicine services,

Electronic health records, and Internet services. Telemedicine is a powerful tool that permits medical experts to be received beyond the boundaries of a medical facility. It saves many of dollars in patient travel costs and missed appointments. However, telehealth isn't new to India and has been around for over a decade now. Health systems across the country have the chance to make a meaningful impact by adopting scalable telemedicine technologies.

Today, we are living healthier and longer lives. Building a healthcare system that effectively manages care for the people is inevitable. Even though we're among the youngest country within the world, this is often a particular state and is merely a matter of time before it becomes a priority. If planned and implemented well, technological progress can make a significant contribution to India's healthcare needs. Such an investment would impact generations to return with lower healthcare costs and put India on the world map as a rustic to be reckoned as an idea leader in adopting digital health.

8.1 LEARNING OUTCOMES

After reading this Unit, you will be able to:

- understand the existing role of technology in shaping rural health and the future ahead;
- define HIS, GIS, telemedicine, e-health; and
- identity the challenges and scope of health related technologies, especially for rural India

8.2 CONCEPTUAL BACKGROUND OF TECHNOLOGY

Technology plays an important role within the maintenance and tracking of service quality. The health information is a crucial tool to enhance the quality, safety, effectiveness, and delivery of healthcare services in rural communities. Health Information System can connect rural patients and providers in remote locations to specialists in urban areas. Implementing, maintaining, updating, and optimizing HIS can be an ongoing challenge for rural facilities and providers with limited resources and expertise. It uses technology to store, secure, retrieve, and transfer protected health information electronically within healthcare systems and community settings. Health information technology is a broad term encompassing a compilation of technologies and programmes to research, house, and share health information.

The health information technology has some important components they include Electronic Health Records (EHR) for patients, rather than paper records, secure digital networks to send and deliver up-to-date records whenever and wherever the patient or clinician may need them, electronic transmittal of medical test results, telehealth applications to increase access to providers, tele monitoring applications that allow patients to transmit vitals or diagnostic information to providers remotely, confidential and secure patient health portals for patients to access their personal health information online, transmission between healthcare providers, also patients, electronic prescribing and ordering to help avoid medical

errors, decision support systems to provide clinicians with information on best practices and treatment options to improve quality of care, mobile devices and tablets to update patient records in real time and document at the point of care. Adoption of technology features a long-term impact on cost, and hence there must be more intentional effort to take a position in technology and adopt this high up-front. It means investing in training and hiring manpower, paying for servers, adoption and implementation costs at the healthcare facilities, which can buy themselves within the future.

In rural areas health information technology works to make sure efficient, coordinated, and secure healthcare information exchange for patients who receive healthcare services from multiple providers or in multiple locations. It allows patients the opportunity to engage in the provision of their healthcare by tracking health conditions, accessing provider visit notes, and test results. The diversity and vastness of the country can pose some of the hardest problems to tackle, but the adoption of technology also comes with its share of challenges. The convergence of technological solutions with data analytics, cloud computing, telecommunications, and wireless technologies will improve accessibility and manage labour shortages more efficiently within the healthcare industry. Benefits derived include easy accessibility regardless of geographical location, fewer errors, fast response for emergencies, and improved patient experience. The cost of providing medical services has also been rising steadily. As technological innovation better incorporates with healthcare delivery, it will enable scale and lower costs, driving up adoption. And adoption are going to be further focused by the automation of critical processes at hospitals in administration, finance, billing, patient records and pharmacies. Policies have always been the most important influencers to implement behavioural change, but technology are often as impactful when it involves changing the way services are delivered.

8.3 HEALTH INFORMATION SYSTEM (HIS)

A Health Information System is a system considered to manage healthcare data and it refers to a system that captures, stores, transmits or then manages health data or activities. These systems are used to collect, process, practice, and report health information. A health information system as the information processing and information storing subsystem of a health care organization, which can be a single institution, for instance, a hospital, or a group of health care institutions sort of a health care network. As an integrated effort, these could also be leveraged to enhance patient outcomes, inform research, and influence policy-making and decision-making. Because health information systems commonly access, process, or maintain large volumes of sensitive data, security is a principal disquiet. Health information technology (HIT) encompasses the development of health information systems. The main functions of a health information system are to monitor, inform and evaluate a health system and to form clinical and management decisions.

HIS for improving the efficiency of any health system, there is a requirement of relevant, timely, and accurate information on its current performance. The purpose of HIS is to provide this information HIS is a tool of management, HIS may be in place at various levels, it may be for one single hospital, a single district, state, and national level and international level. It is defined as a mechanism for the

collection, processing, analysis, and transmission of information required for organizing and operating health services and also for research and training. The primary objective of HIS is to provide reliable, relevant up-to-date, adequate timely, and reasonably complete information for health managers at all levels. Early detection of suspected disease outbreak to enable timely initiation of response efforts, this will reduce the number of cases and deaths during an outbreak. Monitor trends in health status and identify changing health care priorities, the monitoring health status allows health managers to observe trends in the health profile of a population, detect the emergence of new health problems and continually address public health priorities and also evaluating the effectiveness of programmes. The ultimate objective of HIS is not to gain information but to improve action.

Health Information Systems is a broad category that encompasses several specific and here is some of the most common HIS types are: a) Strategic or Operational Systems are typically used for information classification. Provisions are made for information systems based on the type of information they're handling. A pyramid classification system allows organizations to assess the spread of digitization. Because operational systems are generally developed before executive information systems or management information systems, this is easily achieved. The ability to evaluate dependencies can help to identify system deficiencies, as well. For example, a properly configured information system should pull data from a clinical system rather than require nurses and clinicians to collect and document data manually. b) The Clinical and Administrative Systems for Managing Patient Information on an Administrative Level that is the clinical systems are dependent on administrative data. The foundation of an integrated HIS is a master index developed around the most basic patient information with links to different clinical systems, and the clinical system contains the electronic patient record (EPR), diagnostic data, outcomes, and processing. c) Electronic Health Record (EHR) and Patient Health Record, the open EHR aims to enable semantic interoperability for health information systems between various EHR systems in a non-proprietary format to prevent vendor lock-in. Knowledge concepts are stored outside the EHR as archetypes, which support the recording of clinical information. Archetype building blocks include instructions, evaluations, observations, and actions, and information built using these building blocks is stored in the EHR. d) Subject- and Task-Based Systems are related to patients or healthcare professionals in any type of healthcare organization. Task-based systems, on the other hand, are associated with particular tasks such as admission or discharge. Subject-based systems are often preferred, as they reduce data duplication. In a task-based system, the same subject could be related to various tasks, with basic information such as the patient's ID being duplicated across each task. In a subject-based system, this basic information is entered only once and flows with the subject through various tasks. For example, an EHR is a subject-based system. e) Financial and Clinical Health Information Systems these systems provide easy access to patient financial information, such as costs and payors, and they also aid in monitoring patient usage of different departments or services. Financial systems typically include invoicing capabilities as well as tools for following up on non-payment. f) Decision Support Systems convert data to clinically relevant information and present it in the actionable form to clinicians, aiding in adherence to regulatory guidelines and best practices.

These systems can give results for several data manipulations to mimic cognitive processing. For example, a decision support system may provide a list of medications for a particular condition appropriate for the patient's demographics, such as the patient's age and weight, as well as any comorbidities. Decision support systems can also facilitate next step in the workflow, such as submitting a prescription to the pharmacy and scheduling a follow-up appointment for the patient.

Information is crucial at all management levels of the health services from the periphery to the center. It is required by policy makers, managers, health care providers, health workers. The HIS is important to a country because good management is a prerequisite for increasing the efficiency of health services and an improved HIS is clearly linked to good management.

8.3.1 Components of HIS

Health Information System are part of the extensive statistical system, which includes non-health sectors like education and employment. Utmost traditional HIS collect data at a granulated level by various means such as surveys, clinical observation, diagnostic testing or through management and financial information systems. They concentrate on individuals (citizens, patients, health care providers), features of the services they need, use or deliver, the resources required to deliver those services and the impacts that they attain. Those data are then combined, analysed and reported in several ways to generate summary information for use by service providers, managers, planners, researchers, commentators and others with an interest in the health sector. Collecting, analysing and sharing health information is a complex process that requires a clear understanding of its underlying components and how these components interact. The Components and standards of a Health Information System includes the following:

1. HIS resources, such as appropriately trained staff, finance, logistics support and context-specific technologies. These resources (or inputs) must be situated within the broader legislative, regulatory and planning framework of a country.
2. Indicators, the basis of a HIS strategic plan must include a core set of indicators and related targets that can provide a picture of the determinants of health, health system condition, and the status of population health.
3. Data sources, like civil and vital registration (births, deaths and cause-of- death), censuses and surveys, medical records, service records and financial and resource tracking.
4. Data management includes data collection, storage, quality, flow, processing, compilation and analysis.
5. Information products, the transformation of data into information and therefore into a tool for evidence-based decision-making that will lead to improved health.
6. Dissemination and use, increasing the value of health information by making it accessible to decision-makers and providing incentives for the use of health information.

The HIS is composed of several related subsystem which are: 1) Demography and Vital events (D), 2) Environment and health statistic (E), 3) Health Status: mortality, morbidity, disability and quality of life (S), 4) Health Resources: facilities, beds, manpower (R), 5) Utilization and non-utilization of health services: attendance, admissions, waiting lists (U), 6) Indices of Outcome of medical care (O), 7) Financial Statistics (cost, expenditure) related to the particular objective (F).

8.3.2 Evolution

Healthcare information systems that can record and locate important information quickly have become a standard practice in many healthcare organizations. Here summarizing the milestone of development for HIS were considered as important:

(1) the shift from paper-based to computer-based processing and storage, as well as the increase of data in health care settings; (2) the shift from institution centered departmental and, later, hospital information systems towards regional and global HIS; (3) the inclusion of patients and health consumers as HIS users, besides health care professionals and administrators; (4) the use of HIS data not only for patient care and administrative purposes, but also for health care planning as well as clinical and epidemiological research; (5) the shift from focusing mainly on technical HIS problems to those of change management as well as of strategic information management; (6) the shift from mainly alphanumeric data in HIS to images and now also to data on the molecular level; (7) the steady increase of new technologies to be included, now starting to include ubiquitous computing environments and sensor-based technologies for health monitoring.

Eventually the health system is experiencing a paradigm shift, moving from 'Industrial Age Medicine to Information Age Healthcare'. This 'paradigm shift' is affecting healthcare systems and renovating the healthcare patient relationship. For instance, the World Wide Web has changed the way the public engage with health information. Here represents show the world level transitions in the Health information system.

In the 1960s, the main healthcare drivers in this era were Medicare and Medicaid. The IT drivers were expensive mainframes and storage. Because computers and storage were so large and expensive, hospitals typically shared a mainframe. The principal applications emerging in this environment were shared hospital accounting systems.

During the 1970s one of the main healthcare drivers in this era was the need to do a better job communicating between departments (order communications, bed control and results in review) broadened administrative systems, departmental systems processing and the need for discrete departmental systems (e.g., clinical lab, pharmacy). Computers were now small enough to be installed in a single department without environmental controls. As a result, departmental systems proliferated. Unfortunately, these transactional systems, embedded in individual departments, were typically islands unto themselves.

In the 1980s the healthcare drivers were heavily tied to DRGs and reimbursement. For the first time, hospitals needed to pull significant information from both

clinical and financial systems in order to be reimbursed. At the same time, personal computers, widespread, non-traditional software applications, and networking solutions entered the market. As a result, hospitals began integrating applications so financial and clinical systems could talk to each other in a limited way.

The 1990s in this decade, competition and consolidation drove healthcare, along with the need to integrate hospitals, providers, and managed care. From an IT perception, hospitals now had access to broad, distributed computing systems and robust networks with cheaper hardware and storage. Therefore, we created an integrated delivery networks (IDN)–like integration, including the impetus to integrate data and reporting.

The 2000s, the main healthcare drivers have increased integration and the beginnings of outcomes-based reimbursement. We now had enough technology like more of everything Mobility Emerging cloud computers and cloud-based big data analytics and bedside clinical applications installed to make a serious run at commercial, real-time clinical decision support.

As the decades passed, the most commonly implemented systems were those designed to automate transactions, either in a clinical or administrative context. An obvious result of more transaction systems installations was the dramatic increase in readily available digitized data. Another focus of hospital information system implementation over the years has been reporting. Reporting systems typically exist as components of transaction systems. Historically, this reporting provided snapshots of information about the hospital to management, the board, or other groups. Analytics requires more than mere reporting; health systems must support the ability to drill down into this comprehensive, merged data to achieve real insight into operational performance.

Now we find ourselves in the 2010s. The healthcare drivers are accountable care organizations (ACOs) and other value-based purchasing initiatives, a need for cost and quality-control systems, and a broadening genomic influence on personal care. Our main IT driver is pervasive computing. We have microprocessors everywhere. We'll be seeing more and more of them, along with the proliferation of data. IT in the industry has broadly implemented EMRs and operational data systems, and, ultimately, these EMRs will have pervasive clinical decision support. Knowledge gained from analyzing an organization's data in search of performance improvement insights will complete the operational systems cycle by refining the rules essential for successful clinical decision support. These efforts are highly complementary.

The concerned authorities must be actively engaged in a strategy that results in the capture and analysis of comprehensive data, which enables the health system to become an organization that is constantly improving and learning.

Check Your Progress: 1

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) Elaborate the role and components of HIS.

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8.3.3 Indicators

Indicators of country health information system performance can be grouped into two broad types, namely: first one is indicators related to data generation using core sources and methods (health surveys, civil registration, census, facility reporting and health system resource tracking); the second one is indicators related to country capacities for synthesis, analysis and validation of data. Indicators of data generation reflect country capacity to collect relevant data at appropriate intervals and using the most appropriate data sources. Scales include periodicity, timeliness, and contents of data collection efforts and availability of data on key indicators. Indicators of country capacity measure key dimensions of the institutional frameworks needed to ensure data quality including independence, transparency and access. Scales include the availability of independent coordination mechanisms, and the availability of micro data and metadata.

The indicators selected represent various areas of the HIS Strengthening Mode: Some of the Indicators are the following:

- 1) Country has a national health strategy (year),
- 2) Country has a health sector monitoring and evaluation plan (M&E),
- 3) Country has Health Information System policy (year),
- 4) Country has an HIS strategic plan (year),
- 5) Country has set of core health indicators (year updated),
- 6) National HIS coordinating body,
- 7) Country has master facility list (year updated),
- 8) Conducted Health Metrics Network (HMN) assessment (year)
- 9) Population census (within the last 10 years),
- 10) Availability of national health surveys,
- 11) Completeness of vital registration (births and deaths),
- 12) Country has electronic system for aggregating routine facility and/or community service data,
- 13) Country has national statistics office,
- 14) National health statistics report (annual),
- 15) Country's ministry of health has an updated website,
- 16) Data quality assessment (DQA) conducted on prioritized indicators aligned with most recent health sector strategy (year of most recent),
- 17) PRISM assessment conducted in any regions/districts,
- 18) Percentage of facilities represented in HMIS information

- 19) Proportion (facility, district, national) offices using data for setting targets and monitoring,
- 20) Measles coverage reported to World Health Organization (WHO)/UNICEF,
- 21) Number of institutional deliveries (births) available by district and published within 12 months of preceding year,
- 22) Existence of policies, laws, and regulations mandating public and private health facilities/ providers to report indicators determined by the national HIS,
- 23) Availability of standards/guidelines for RHIS data collection, reporting, and analysis,
- 24) Presence of procedures to verify the quality of data (accuracy, completeness, timeliness) reported,
- 25) RHIS data collection forms allow for disaggregation by gender,
- 26) At least one national health account completed in last five years,
- 27) National database with health workers by district and main cadres updated within the last two years,
- 28) Annual data on availability of tracer medicines and commodities in public and private health facilities,
- 29) e- Health strategy,
- 30) Completeness of disease surveillance reporting.

8.3.4 Uses and Purpose

The HIS is mainly used to measure the health status of the people and to quantify their health problems and medical and health care needs. It is used for local, national and international comparisons of health status, for such comparisons the data need to be subjected to rigorous standardization and quality control. For planning, administration and effective management of health services and programs and for assessing whether health services are accomplishing their objective in terms of their effectiveness and deficiency or evaluating the health services. And also the HIS is using for research into particular problems of health and disease.

Health information underpins the entire health system: it strengthens stewardship, can be used in strategic planning and priority-setting, as well as within clinical diagnosis and management, quality assurance and improvements, and global epidemics. Healthcare information promotes excellence in care; describes the types of people using a service and the types of services received; helps coordinate services; provides meaningful information on the health status of the community; and ensures accountability. The resolution calls upon the need to identify evidence-based cost-efficient interventions, and strengthened monitoring and evaluation systems that, 'are integrated into existing national health information systems and include the monitoring of risk factors, outcomes, social and economic determinants of health, and health systems responses'. Information about the functioning of the health information system can be obtained from the different

sectors and agencies that have responsibilities for the generation, synthesis, analysis and use of data at country, regional and global levels.

At country level, Ministries of Health record the timeliness and quality of data reported through health services and disease surveillance systems. National Statistics Offices maintain information about the availability and quality of data generated through major data collection undertaking such as the decennial census, large scale household surveys, and the civil registration system. As custodians of national official statistics, they often have explicit requirements for the way data are collected, compiled and shared, and adhere to the Fundamental Principles of Official Statistics. International agencies working in health also maintain information about the availability and quality of data on international health goals, including but not limited to the Millennium Development Goals or Sustainable Development Goals.

The healthcare industry relies on a massive amount of data to make decisions about patient care, facilitate the delivery of care, and handle the many complex administrative tasks that go on behind the scenes. Health information systems are valuable tools that aid clinicians and administrative personnel in ensuring a seamless patient experience from end-to-end. Additional purpose include: Data analytics, the HIS help to gather and analyze data to manage population health and reduce healthcare costs. Supports collaborative care, the HIS facilitates the sharing of PHI between providers and organizations, making it possible for patients to receive coordinated care from multiple providers while improving care delivery and patient outcomes. Cost control, by sharing information, HIS can eliminate duplicate testing and procedures, reduce time demands on staff (such as for sending paper copies of patient records), and reduce costly human errors. Population health management, aggregating patient data can help to identify patterns and trends, predict or prevent outbreaks, identify at-risk populations, and more. Clinical decision support, integrating a patient's individual data and medical history with broader population data and research improves both diagnostics and treatment.

Check Your Progress: 2

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) How can HIS contribute in achieving Sustainable Development Goal of Health for all?

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Examples

Health information systems can be used by everyone in healthcare from patients to clinicians to public health officials. They collect data and compile it in a way that can be used to make healthcare decisions. The health information systems run the extent from high-level administrative systems to those that manage detailed, patient-specific information. Some of the Examples of health information

systems include: **Electronic Medical Record (EMR) and Electronic Health Record (EHR)**: These two terms are almost used interchangeably. The electronic medical record replaces the paper version of a patient's medical history. The electronic health record includes more health data, test results, and treatments. It also is designed to share data with other electronic health records so other healthcare providers can access a patient's healthcare data. **Practice Management Software**: It helps healthcare providers manage daily operations such as scheduling and billing. Healthcare providers, from small practices to hospitals, use practice management systems to automate many of the administrative tasks. **Master Patient Index (MPI)**: It connects separate patient records across databases. The index has a record for each patient that is registered at a healthcare organization and indexes all other records for that patient. MPIs are used to reduce duplicate patient records and inaccurate patient information that can lead to claim denials. **Patient Portals**: It allow patients to access their personal health data such as appointment information, medications and lab results over an internet connection. Some patient portals allow active communication with their physicians, prescription refill requests, and the ability to schedule appointments. **Remote Patient Monitoring (RPM)**: Also known as telehealth, remote patient monitoring allows medical sensors to send patient data to healthcare professionals. It frequently monitors blood glucose levels and blood pressure for patients with chronic conditions. The data is used to detect medical events that require intervention and can possibly become part of a larger population health study. **Clinical Decision Support (CDS)**: It's a system to analyze data from various clinical and administrative systems to help healthcare providers make clinical decisions. The data can help prepare diagnoses or predict medical events like medicine interactions. These tools filter data and information to help clinicians care for individual patients.

There are many specific health information systems, some of the Specific examples include: Master Patient Index (MPI), Medical billing software, Patient portals, Health Information Exchange (HIE), Activity Based Costing (ABC), Patient Reported Outcomes (PRO), Remote Patient Monitoring (RPM), Scheduling software, e-Prescribing software, Laboratory information systems, Hospital Patient Administration Systems (PAS), Human Resource Management Information Systems (HRMIS).

8.4 CLINICAL INFORMATION SYSTEM

A Clinical Information System (CIS) is a computer-based system that is designed for collecting, storing, manipulating, and making available clinical information important to the healthcare delivery process. It may be limited in the extent to a single area (e.g. laboratory systems, ECG management systems) or they may be more widespread and include virtually all aspects of clinical information (e.g. electronic medical records). The CIS provides a clinical data repository that stores clinical data such as the patient's history of illness and the interactions with care providers. The repository encodes information capable of helping physicians decide about the patient's condition, treatment options, and wellness activities also because the status of selections, actions undertaken, and other relevant information that would help in performing those actions.

The CIS is a computer-based platform that permits healthcare providers and administrators to collect, store, manage, and retrieve clinical information and patient data. The system is made of building blocks that address various areas like the Electronic Medical Records (EMRs), which stores patient's personal details and medical record. Clinical Decision Support is a tool that aids physicians in making accurate evidence-based clinical decisions, and even training and research, a platform that trains doctors.

The system are often used within a selected perimeter, such as within one laboratory, or just for management of all clinical imaging like x-rays and MRIs, but also can be extended and integrated coverage across departments, ultimately improving the interaction between doctors, nurses, and allied health through integrated healthcare, and resulting in better provision of look after patients. The CIS is meant to permit for structured organization of data and automated reports, like for patient fees and bills, pharmaceutical inventories, management of medicine approaching expiration dates, insurance reports, and even reports of adverse drug reactions to be submitted to the Drug Control Authority. The ICT based CIS reduces paperwork, which saves costs and cuts the necessity for physical space for storing, especially in countries where it's legally authorised for healthcare information to be stored for a minimum number of years.

CISs are crucial to delivering the simplest in evidence-based, and patient-centered care. It has great potential in reducing medical errors, increasing legibility, cutting unnecessary healthcare costs, and boosting the standard of healthcare. The main role of CISs is to capture, store, process, and timely transfer information to clinical decision-makers or an accurate and rapid decision. For instance, a CIS can easily import data from different instruments like vital signs monitors, ventilators, and infusion devices, store them safely and display them in specific tables and formats.

One advantage of this sort of system is to interrelate with other subsystems within the hospital, e.g. pharmacy, different laboratories, radiology, and different image processing storage solutions. A decent CIS pays positively to patient safety, workflow efficiency, and point-of-care decision support. The progress of CISs has posed some new challenges and, at an equivalent time, has also generated new opportunities.

CISs clearly offer excellent opportunities for improving care quality. Nevertheless, implementing CISs in healthcare organizations poses a series of challenges. The adoption of IT in healthcare has been particularly slow and lagging behind as compared to other domains. This is because of the complexity of issues like interoperability, technological rationality, acceptability, managerial rationality, data security, data quality, and standards. But more significantly, the CIS improves patient safety in some ways. Information is well organized, legible, and reduces the likelihood of medical error owing to illegible writing, enables safer and more reliable and prescribing, and thus eases communication between allied health professionals. Patient information is additionally secure and can be safely united with patients and other clinicians when necessary, enhancing continuity of care for patients. Eventually, the improved efficiency helps to enhance productivity and optimize profits within the clinic or hospital, and if integrated across all facilities, can eventually improve the healthcare system as an entire. Healthcare organizations should adopt CISs to expand the excellence of care and to be ready

to stay competitive. The fundamental goal is to strike a balance between available resources, current HIS architecture, and therefore the desired clinical improvement objectives.

8.5 GEOGRAPHIC INFORMATION SYSTEM (GIS)

Health is vital for all of us and understanding the causes of a disease, its spread from person to person and community to community has become increasingly global. Since health is a geographical phenomenon and various factors attributing to the health diagnostics and planning are geography dependent, as such, GIS (Geographic Information System) for health studies assists as an important tool. GIS can be useful for health researchers and planners because it plays a vital role in strengthening the whole process of epidemiological surveillance, information management and analysis. It aids as a common platform for convergence of multi-disease surveillance activities and it is being used by public health administrators and professionals, including policy makers, statisticians, regional and district medical officers.

GIS data sets can be useful for many purposes. They are used primarily to display data geographically. In public health, applications for GIS are becoming more accessible as geo-coded health data and environmental exposure data increasingly become available, and new and easier-to-use GIS software is developed. It includes database management, planning, risk service area mapping, location identification, etc. GIS (Geographic Information System) is a useful tool that aids and assists in health research, health education, planning, monitoring and evaluation of health programmes that are meant to control and eradicate certain life threatening diseases and epidemics.

Some of its applications in public health are: (1) geographical distribution and variation of diseases (2) analysis of spatial and temporal trends (3) identifying gaps in immunizations (4) mapping populations at risk and stratifying risk factors (5) documenting health care needs of a community and assessing resource allocations (6) forecasting epidemics (7) planning and targeting interventions (8) monitoring diseases and interventions over time (9) managing patient care environments, materials, supplies and human resources (10) monitoring the utilization of health centers (11) route health workers, equipment's and supplies to service locations (12) publishing health information using maps, etc.

The development of methods for mapping disease occurrence has advanced noticeably in recent years. This growth in interest has led to a larger use of geographical or spatial statistical tools in the analysis of data both routinely collected for public health purposes and in the analysis of data found within ecological studies of disease relating to explanatory variables. The study of geographical distribution of diseases can have a multiplicity of uses and can fit into any of the three classes: Disease mapping, usually the object of the analysis is to estimate the true relative risk of a disease of interest across a geographical area. Application of such methods lies in health service resource allocation. Disease clustering, this aids in public health surveillance, to adopt where it may be important to be able to assess whether a disease map is clustered and where

the clusters are located. The analysis of disease incidence around an assumed source of hazard is a special case of cluster detection. Ecological analysis, this focuses on the analysis of the geographical distribution of disease in relation to explanatory covariates, usually at an aggregated spatial level.

Check Your Progress: 3

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) Briefly discuss the application of GIS in health planning?

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8.6 HEALTH STATISTICS

The health statistics are derived from data sets which are collections of logically related data arranged during a prescribed manner. Data is a piece of information, discrete observations of attributes or events, especially people who are a part of the collection to be analysed. Data that is accurate, timely, specific, and organized for a purpose, presented within a context that provides it meaning and relevance and may cause a rise in understanding and reduce in uncertainty. Health statistics and data are important because they measure a good range of health indicators for a community. Health data provide comparisons for clinical studies, are often wont to assess the prices of health care.

The general principles of the science of health statistics, as of most others, were foreshadowed by the ancient Greeks, who carefully observed, recorded and analyzed natural phenomena, and applied the inductive method of reasoning in a search for natural rather than supernatural causes, treating the unproven with scepticism. Nevertheless, the proper understanding of the behaviour of disease—how and why disease in a human (or animal) population spreads, waxes and wanes, or alters its character in response to the ceaseless changes that take place in its host and environment—had to await the techniques of study and experiment provided by modern epidemiology, statistics, microbiology, and biochemistry.

Health statistics are numbers that summarize information related to health. It is a form of evidence or facts that can support a conclusion. Evidence-informed policy-making, an approach to policy decisions that are intended to ensure that decision making is well-informed by the best available research evidence, and evidence-based medicine (EBM), or the conscientious, explicit, judicious, and reasonable use of modern, best evidence in making decisions about the care of individual patients are essential to informing how best to provide health care and promote population health. Not all evidence is or should be, equally convincing in the support of a conclusion. Evidence varies in quality and whether it is applicable to a given situation. It is therefore essential that health researchers and policymakers understand how to assess evidence in a systematic way, including how to access transparent, high-quality health statistics and information. Health statistics measure four types of information. The types are commonly referred to

as the *four Cs: Correlates, Conditions, Care, and Costs*. The first section of this course examines each type of information.

Basic terms and concepts used to collect and present data are essential to using health statistics and to assess their quality. Some of the basics of statistical research as it relates to health care, which includes sampling, confidence intervals, bias, validity, dependent and independent variables, age adjustment (including direct standardization), and more. Sampling: Take a look at how a small group of people can help us estimate information about a large population, Confidence Intervals: See how to identify how accurate certain data is, Correlation and Causation: Differentiate between two different types of relationships between variables, Validity: Find out how to measure the accuracy of statistics, Dependent and Independent Variables: Identify the different ways variables relate to the people being studied, Age Adjustment: Learn how to adapt statistics to account for age differences between communities or groups, Hypotheses: Understand the language behind experiment design, Normal Distribution: Visualize how data is normally distributed in populations, Standard Deviation: Learn about the metric used to describe dispersion of data, Z-score : Learn a formula to determine a data point's position along a normal distribution curve in relation to the sample mean, t-test and Z-test : Determine statistical significance using these statistical tests, Significance Level : Understand how researchers conceptualize statistical significance, p-value: The final step, learn the concept behind probability of error, Incorporation into Health Subjects.

Health statistics are population based and many are collected and analyze over time. Statistics often use geographic regions such as zip codes for determining health care coverage and comparisons of specific disease occurrences. Most studies focus on variation over time, space and social group. Researchers and experts from government, private, and non-profit agencies and organizations collect health statistics. They use the statistics to learn about public health and health care.

8.6.1 Purpose and Sources

Statistics starts with numbers that are universally not fixed. The numbers are put together (in addition, subtraction, multiplication, and division) and conclusions are drawn. Health statistics are used to understand risk factors for communities, track and monitor diseases, see the impact of policy changes, and assess the quality and safety of health care. However, to be of use statistics must be both reliable and relevant. They need to be compiled correctly, following standard practices and methodology. They must also meet the needs of users. The ultimate objective of collecting data is to inform health programme planning as well as policy-making and, finally, global health outcomes and equity. Well-functioning health information systems empower decision-makers to manage and lead more effectively, by providing valuable evidence at the lowest possible cost.

By and large, the purpose of vital and health statistics is to contribute to social and economic progress. Prevailing levels of health and demographic problems have to be studied in relation to the situations and factors that influence them. For this—and for the study of trends and changes in health, conditions—factual knowledge is essential. The more advanced a country, the more accurate and

penetrating are the studies it has to undertake. For instance, such basic steps as the improvement of environmental sanitation, the establishment of maternal and child health services, and health education-to name only a few-may well lead full statistical analysis of the situation. Nevertheless, even simple varieties of vital and health statistics, such as those which help in locating areas of high mortality and indicating the major preventable diseases, would be helpful. These can be produced relatively easily, either by means of sample surveys or by the establishment of a national health statistics system. The standard statistical methods for collecting data are not uniformly applicable throughout the world.

Health Statistics come from diverse sources many countries have their own systems for reporting data. The main sources of health statistics are surveys, administrative and medical records, claims data, vital records, surveillance, disease registries, and peer-reviewed literature. Depending on the measure, data can be collected from different sources, including medical records, patient surveys, and administrative databases used to pay bills or to manage care. Each of these sources may have other primary purposes, so there are advantages and challenges when they are used for the purposes of quality measurement and reporting. The organizations generate administrative data on the characteristics of the population they serve as well as their use of services and charges for those services, often at the level of individual users. The data is gathered from claims, encounters, enrolment, and provider systems. Common data elements include the type of service, number of units (e.g., days of service), diagnosis and procedure codes for clinical services, location of service, and amount billed and the amount refunded, available electronically, less expensive than obtaining medical record data, available for an entire population of patients and across payers, fairly uniform coding systems and practices are the main advantages of the administrative data. It also has some challenges such as limited clinical information, questionable accuracy for public reporting because the primary purpose is billing, completeness, and timeliness.

A medical record is documentation of a patient's medical history and care. The advent of electronic medical records has increased the accessibility of patients' files. Wider use of electronic medical record systems is expected to improve the ease and cost of using this information for quality measurement and reporting. It has some notable advantages such as rich in clinical detail and viewed by providers as credible. The challenges of medical records include the cost, complexity, and time required to compile data when patients receive services across different sites, particularly if a different record format is used and current use of paper for most records, which means that trained staff must manually abstract information.

Survey instruments capture self-reported information from patients about their health care experiences. Aspects covered include reports on the care, service, or treatment received and perceptions of the outcomes of care. Surveys are typically administered to a sample of patients by mail, by telephone, or via the Internet. Captures types of information for which patients are the best source, well-established methods for survey design and administration, easy for consumers to understand and relate to survey results these are the advantages of the patient surveys. The challenges of patient surveys were the cost of survey administration and possibility of misleading results if questions are worded poorly, survey administration procedures are not standardized, the population sampled is not

representative of the population as a whole (sampling bias), or the population is not represented in the responses (response bias).

For Standardized Clinical Data a certain kinds of facilities, such as nursing homes and home health agencies, are required to report detailed information about the status of each patient at set time intervals. The Minimum Data Set (MDS), the required information for nursing homes, and the Outcome and Assessment Information Set (OASIS), the data required by Medicare for certified home health agencies, store the data used in quality measures for these provider types. The advantages of Standardized Clinical Data include uses existing data sets and characterizes facility performance in multiple domains of care. The major challenge of Standardized Clinical Data is it may not address all topics of interest.

International Data United Nations Statistics Division; It compiles statistics from many international sources and produces global updates in specialized fields of statistics. Free access is provided to country-specific population data. World Bank Group Data and Statistics—provides data derived, either directly or indirectly, from official statistical systems organized and financed by national governments. Click on Data by topic to find several options including health. World Health Organization – Statistical Information System (WHOSIS), The WHO Statistical Information System is the guide to health and the health-related epidemiological and statistical information available from the World Health Organization. International health statistics provide the necessary world information system. They are also made use of by various countries to compare their progress in health with that achieved by similar health action elsewhere. Because of these national requirements, international agencies have the unique responsibility of collecting and promptly disseminating statistical information on a worldwide basis.

8.7 PLANNING FOR RURAL HEALTH USING GIS, HIS AND HEALTH STATISTICS: SOME EXAMPLES

Geographic Information System (GIS) provide as a common platform for the convergence of multi-disease surveillance activities. Public health resources, specific diseases and other health issues can be mapped in relation to their surroundings and existing health and community infrastructures. GIS helps to generate thematic maps that render the intensity of a disease. GIS aids in faster and better health mapping and analysis than the conventional methods. Using GIS, helps those working in the health industry manage their information to make better decision. By tying information to specific locations, including hospital and surgeries, contaminated land and spread of disease, analysts can produce maps that help them identify patterns and understand relationships. GIS provides a common analytical framework within which health authorities can understand problems and formulate responses, improving incident management and health planning. GIS is becoming a vital tool in healthcare applications. It includes database management, planning, risk service area mapping and location identification, etc.

Geographical Information System (GIS) in a Health Management Information System (HMIS) can be a powerful tool to make health care delivery more effective

and far more efficient. It includes database management, planning, risk service area mapping, location identification, etc. One of the causes for this sudden surge of GIS use in healthcare application is the spatial dependency of health related factors. Sikkim, a small state in the Eastern Himalayas has shown, in recent times an obvious increase in the total registered deaths of an emergent nature, in the form of cardiovascular, accident and other emergency situations. The existing system is at best, inadequate in terms of equipment and skilled human-power to deliver Emergency Medical Services. STNM Hospital, the central referral institution of the state which provides Emergency Medical Care not only to the urban population of the state capital of Gangtok but also to most of the rural population of the rest of Sikkim. The alarming increase in the workload due to faster development and always rising vehicular traffic and frequent natural calamities calls for the need to expand the facilities. Compounding the problem of the lack of proper management facilities and prompt attention to emergency patients is the hilly terrain where invariably the only possible mode of transport is by road, which in the monsoon season is time and again breached, making transportation of patients a major problem. This study was carried out to introduce the general position of the Gangtok area through the Health Information and Medical preparedness using GIS study. Mapping of essential resources like road networks, locate the health facility in the study area and find out the population density using GIS techniques. Health management requirements, information on a different phase like the occurrence of diseases, facilities that are available in arranging to take decisions on either creating infrastructure facilities for taking immediate action to handle the situation and quickly. Medical preparedness plan requires an integrated approach to disaster site management and clinical casualty management working in a coordinated technically. Medical and health response assumes the top most priority in the entire task of disaster management. GIS is an ideal tool for planning, organizational issues of health service. Medical preparedness, according to the Medical Preparedness for Disaster, implies local reinforcement of important medical care functions. It is invoked when available capacity is or can be expected to be insufficient for emergency care. Medical preparedness basically implies planning for medical and health related aspects for meeting disasters.

Hospital Information Systems Implementation: An Evaluation of Critical Success Factors in Northeast of Iran the implementation of hospital information systems (HIS) is considered as a difficult and sensitive task in terms of its scope and its mission to collect identity-related, demographic, clinical and managerial data of patients in an integrative manner as well as due to the changes it makes in users' working practices. Ever-increasing development of technologies and strong desire to use computers among different user have driven medical centers towards using information systems. Given the large amount of clinical data generated by medical centers and the necessity to access such data, manual hospital information systems (HIS) has encountered numerous problems in a way that access to comprehensive information is not possible. It should be noted that successful implementation of a system at hospitals requires a series of issues and measures in a formulated and relatively long-term program. The success or failure of HIS implementation in hospital's organization are muchelaborated. For this complexity HIS implementationis not similar to other sophisticated technology. Also successful implementation

of HIS is affected by numerous factors. Among the most important ones were functional factors, project management factors; and human, technical, and management factors.

Check Your Progress: 4

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) What is the role of health technology in managing Covid 19 crisis in India?

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8.8 LET US SUM UP

Around the world, the health status of people living in rural and remote areas is generally worse than that seen in people living in urban areas. Rural areas consistently have lower levels of access to primary health care services, safe water and sanitation. Eighty per cent of the poor in Latin America, 60 per cent in Asia and 50 per cent in Africa live on marginal lands of low productivity and high susceptibility to degradation. Poverty and poor health are found more commonly in the world's rural people than in its urban residents. Achieving good health is a well-proven path out of rural poverty. In order to achieve good health in rural places, several key principles must be applied. Society needs to reaffirm its commitment to primary health care based systems, and resources must be distributed in ways that support rural, as well as urban sites. We must efficiently utilize existing facilities in ways that promote high quality services. Individual (personal) health care systems must be closely integrated with public (population based) health systems.

In an ideal world, we would have more real-time data on incidence of community level disease prevalence and would be able to do so much to improve the quality of life for these communities, but the opportunity to do so still stands before us. The ongoing implementation, maturation, and use of electronic health information systems require a complementary and overlapping skill set of competent HIM and HI professionals to support these systems in practice, making it difficult to continue to separate the two professions as distinctly different.

As the healthcare sector in India becomes more prominent in terms of its revenue generation and its need for a strong labour force, innovation in the sector is not far behind. Building infrastructure so as to leverage data to make healthcare decisions can be a powerful endeavour. Community level data analysed correctly could help predict the level of incidence of disease and offer opportunities for preventative healthcare in the medium to long term. Though more challenging to implement, electronic medical record systems is another big opportunity for India to leverage. This makes healthcare more efficient, safer, and cost effective over time. Having everyone's medical records electronically saved has far-reaching impact that outweighs the heavy cost of this transition. India, today, is not far behind to adopt electronic record keeping in medical facilities and is further along the pathway of digitising healthcare records.

The Aadhaar card system can be the catalyst to a strong electronic health record system in India. Even though Aadhaar today focuses much on linking income taxes, passports, bank accounts etc., this will only serve as a foundation for a stronger medical record system. Some of the biggest benefits of having access to electronic health data through an EMR is being able to mine the data at a larger community level and understand the incidence of chronic diseases. For example, understanding the prevalence of diabetes and the need to set up dialysis centers in otherwise out of reach communities in India is invaluable. Similarly, malnutrition cases among children who live below the poverty line can be analysed based on location and specific measures can be taken to help those in need.

The private sector needs to be involved in a greater integration and possible exchange of patient data. Ideally, the patient should administer and own their own health data and they themselves allow for the access to their information by health care institutions, be it the public or private system, but in Brazil, there is still a long way to go as far as the integration of HIS, a hindering factor to this is the digital exclusion, the Brazilian territory's immense length, the need to create integration and exchange of data programs, and investments, among other things.

Efforts to improve health information systems cannot fall on resource-limited countries alone, or on only a few development partners. A programme for reform and improvements must build on country ownership and efforts, and increase the demand for and supply and use of information. Healthcare providers have reliable and multifaceted information from several authoritative sources through their Internet connections. Within the reach of the mouse and keyboard, these rich resources can illuminate a clinician's local experiences, offer comparisons in healthcare delivery and outcomes, and provide validated evidence-based practices.

Few countries have sufficiently effective health information systems even to adequately monitor the Millennium Development Goals. There has been a chronic under-investment in systems for data collection, analysis, dissemination and use; and even when data are available, they are often out of date and unreliable. Efforts to improve health information systems cannot fall on resource-limited countries alone, or on only a few development partners. A programme for reform and improvements must build on country ownership and efforts, and increase the demand for and supply and use of information.

With the concentration of poverty, low health status and high burden of disease in rural areas, there is a need to focus specifically on improving the health of people in rural and remote areas, particularly if the urban drift is to be slowed.

8.9 KEY WORDS

E-health is termed as any electronic exchange of health-related data through electronic connectivity for enlightening competence and effectiveness of health care delivery. The solutions that are provided through e-health initiatives within hospitals include Hospital Information Systems (HIS), telemedicine services, Electronic health records, and Internet services.

Telemedicine is a powerful tool that permits medical experts to be received beyond the boundaries of a medical facility.

Health Information Technology is a broad term encompassing a compilation of technologies and programs to research, house, and share health information. Health information technology (HIT) encompasses the development of health information systems. The main functions of a health information system are to monitor, inform and evaluate a health system and to form clinical and management decisions.

Health Information System is a system considered to manage healthcare data and it refers to a system that captures, stores, transmits or then manages health data or activities. These systems are used to collect, process, practice, and report health information.

Clinical Information System (CIS) is a computer-based system that is designed for collecting, storing, manipulating, and making available clinical information important to the healthcare delivery process.

Geographical Information System (GIS) is a system designed to capture, store, manipulate, analyze, manage, and present all types of geographical data.

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8.11 CHECK YOUR PROGRESS: POSSIBLE ANSWERS

Check Your Progress: 1

1. Refer to Section 8.3.1

Check Your Progress: 2

1. Refer to Section 8.3.3

Check Your Progress: 3

1. Refer to Section 8.5

Check Your Progress: 4

1. Refer to Section 8.7

UNIT 9 GOVERNMENTAL AND NON-GOVERNMENTAL INITIATIVES

Structure

- 9.0 Introduction
- 9.1 Learning Outcomes
- 9.2 Governmental Healthcare Initiatives
 - 9.2.1 Obamacare
 - 9.2.2 Governmental Healthcare Initiatives in India
 - 9.2.3 Role of Government of India in Preservation and Promotion of Public Health
 - 9.2.4 Healthcare Schemes in India
- 9.3 Non-Governmental Initiatives
 - 9.3.1 Non-Governmental Organizations in Healthcare System
 - 9.3.2 Global and Local Perspectives
 - 9.3.3 Non-Governmental Global Healthcare Initiatives
 - 9.3.4 Non-Governmental Healthcare Organizations in India
- 9.4 Let Us Sum Up
- 9.5 Key Words
- 9.6 Further Readings
- 9.7 Check Your Progress: Possible Answers

9.0 INTRODUCTION

Health is a fundamental human right and a global social goal. It is pertinent for the realization of basic human needs and for a better quality of life. As the world and its economies become increasingly globalized, including extensive international travel and commerce, it is necessary to think about health in a global context. Rarely a week goes by without a headline about the emergence or re-emergence of an infectious disease or other health threat somewhere in the world.

Globally, the rate of deaths from non-communicable causes, such as heart disease, stroke, and injuries, is growing. At the same time, the number of deaths from infectious diseases, such as malaria, tuberculosis, and vaccine-preventable diseases, is decreasing. Many developing countries must now deal with a 'dual burden' of disease: they must continue to prevent and control infectious diseases, while also addressing the health threats from non-communicable diseases and environmental health risks. As social and economic conditions in developing countries change and their health systems and surveillance improve, more focus will be needed to address non-communicable diseases, mental health, substance abuse disorders, and, especially, injuries. Some countries are beginning to establish

programs to address these issues. For example, Kenya has implemented programs for road traffic safety and violence prevention.

Additionally, expanding international trade introduces new health risks. A complex international distribution chain has resulted in potential international outbreaks due to foodborne infections, poor quality pharmaceuticals, and contaminated consumer goods. The world community is finding better ways to confront major health threats. Melting geographic boundaries with easy and frequent travel of a large number of people and goods across nations, have posed, newer challenges and health risks.

9.1 LEARNING OUTCOMES

After reading this Unit, you should be able to:

- explain the role of government organizations holistic healthcare;
- describe the governmental healthcare projects and initiatives in India;
- explain the role of non-governmental organizations and understanding of the working of social sector;
- describe non-governmental healthcare initiatives in India; and
- critically analyse how government and non-government organizations can work together for 'Health For All'.

9.2 GOVERNMENTAL HEALTHCARE INITIATIVES

Health systems are grappling with the effects of existing communicable and non-communicable diseases and also with the increasing burden of emerging and re-emerging diseases. The causes of health inequalities lie in the social, economic and political mechanisms that lead to social stratification according to income, education, occupation, gender and race or ethnicity. Health is a causative factor that affects country's aggregate level of economic growth. Since development is a consequence of good health, the governments of even the poorest developing countries should make it a priority to invest in the health sector.

9.2.1 Obamacare

Obamacare is the Patient Protection and Affordable Care Act (ACA) that has changed the way the United States delivered health care overall. The term 'Obamacare' was first coined by critics of the former president's efforts to reform health care, but then, the name stuck.

Before the ACA, insurance companies could exclude people with pre-existing conditions. As a result, the people with the greatest health expenses sometimes had to go without insurance or settle for a policy that did not cover a pre-existing condition. Because they couldn't afford regular doctor visits, they often ended up in hospital emergency rooms and unable to contribute to the expense of their treatments. The ACA's primary goal was to slow the rising cost of health care by taking steps to make health insurance more available and more affordable to those who need it the most. The act also required everyone to carry health insurance or pay a tax penalty.

A notable part of the Affordable Care Act was the individual mandate, a provision requiring all Americans to have healthcare coverage – either from an employer or through the ACA or another source – or face increasingly stiff tax penalties. This mandate served the double purpose of extending healthcare to uninsured Americans and ensuring that there was a sufficiently broad pool of insured individuals to support health-insurance pay-outs.

The Act was signed into law in March 2010, designed to extend health insurance coverage to millions of uninsured Americans. Although Congress made major changes to Obamacare, the ACA still remains strongly in place. Changes have been made to the law that have addressed some of the objections raised by opponents, while still keeping the marketplace open active for users. The Act expanded Medicaid eligibility, created health insurance exchanges, and prevented insurance companies from denying coverage or charging more due to pre-existing conditions. The Act expanded Medicaid eligibility and created a Health Insurance Marketplace. It prevents insurance companies from denying coverage due to pre-existing conditions and requires plans to cover a list of essential health benefits.

Check Your Progress: 1

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) 'Obamacare initiative has been a radical step in delivering health care services'. Justify this statement.

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9.2.2 Governmental Healthcare Initiatives in India

India is one of the fastest growing economies of the world. India presently is the second-largest populated nation comprising of 18 per cent of the world's population. This is approximately combined population of the six countries, namely the USA, Indonesia, Brazil, Pakistan, Bangladesh and Japan. However, with large human resource base comes many inherent challenges. India is bearing the dual burden of diseases where on the lower end, malnutrition, hygiene, immunization, sanitation and infectious diseases are major concerns; and on the higher end, environmental health and lifestyle diseases and other non-communicable diseases have raised alarm. Cardiovascular diseases, tuberculosis, cancer, diabetics, malaria, dengue fever, chikungunya, respiratory infections, vector and water-borne diseases continue to be major challenges among the latter group. The very essential components of primary health care – promotion of food supply, proper nutrition, safe water and basic sanitation and provision for quality health information concerning the prevailing health problems – is often largely ignored. Access to healthcare services, provision of essential medicines and scarcity of doctors are other bottlenecks in the primary health care scenario.

The practice of public health has been dynamic in India, and has witnessed many hurdles in its attempt to affect the lives of the people of this country. Since independence, major public health problems like malaria, tuberculosis, leprosy, high maternal and child mortality and lately, Human Immunodeficiency Virus (HIV) have been addressed through a concerted action of the government and other agencies. Social development coupled with scientific advances and health care has led to a decrease in the mortality rates and birth rates.

Healthcare has become one of India's largest sector, both in terms of revenue and employment. Healthcare comprises hospitals, medical devices, clinical trials, outsourcing, telemedicine, medical tourism, health insurance and medical equipment. The Indian healthcare sector is growing at a brisk pace due to its strengthening coverage, services and increasing expenditure by public as well private players. The new agenda for Public Health in India includes the epidemiological transition (rising burden of chronic non-communicable diseases), demographic transition (increasing elderly population) and environmental changes. The unfinished agenda of maternal and child mortality, HIV/AIDS pandemic and other communicable diseases still exerts immense strain on the overstretched health systems.

School health, mental health, referral system and urban health remain as weak links in India's health system, despite featuring in the national health policy. School health programs have become almost defunct because of administrative, managerial and logistic problems. Mental health has remained elusive even after implementing the National Mental Health Program.

On a positive note, innovative schemes through public-private partnerships are being tried in various parts of the country in promoting referrals. Similarly, the much awaited National Urban Health Mission might offer solutions with regards to urban health.

The health sector in India is public, government, private or individual owned. Private sector healthcare providers, registered under the Clinical Establishment Act, are owned and run by individuals or a group of individuals. Public sector, on the other hand, comes under the Ministry of Health and Family Welfare, Government of India. They too consist of dispensaries, clinics, nursing homes and hospitals that follow various kinds of medicine systems. Additionally, it includes all India networks of government health facilities in the form of sub-centres, primary health centres, community health centres and rural hospitalizing, urban health centres, municipal and other government hospitals. Charitable institutions, religious organisations like churches and NGOs and public sector bodies like atomic energy, railways, port trust, reserve bank and armed forces also own many of these also.

9.2.3 Role of Government of India in Preservation and Promotion of Public Health

Public health is concerned with disease prevention and control at the population level, through organized efforts and informed choices of society, organizations, public and private communities and individuals. However, the role of government is crucial for addressing these challenges and achieving health equity. The Ministry

of Health and Family Welfare plays a key role in guiding India's public health system, being responsible for the implementation of various programmes related to health and family welfare, prevention and control of major communicable diseases and promotion of traditional and indigenous systems of medicines. It also undertakes research, provides technical assistance and implementation of World Bank-assisted programmes like control of malaria, tuberculosis, AIDS, among others. Programmes having implications at the national level come under the concurrent list like family welfare and population control, medical education and prevention of food adulteration. Public health, hospitals, dispensaries and sanitation fall under the State list.

The central government lays down the framework and provides direction to all programmes to be undertaken like smallpox, malaria, tuberculosis, HIV/AIDS, leprosy and others, implemented across the country uniformly. It is responsible to provide funds to the state government for implementation and execution of all the initiatives. The states also implement all centrally funded programmes like family planning, *Swachh Bharat Abhiyan* and universal immunization. Since good health and wellbeing have overlapped with various other dimensions, many ministries together have to work for the promotion of healthcare facilities. Various ministries that directly or indirectly contribute towards good health of the Indian population. Of the total 58 ministries, 26 are related to provision of healthcare services and promotion of good health in the country.

9.2.4 Healthcare Schemes in India

Some of the major initiatives taken by the Government of India to promote Indian healthcare industry are as follows:

- In the Union Budget 2020-21, Rs 35,600 crore has been allocated for nutrition-related programmes and Rs 69,000 crore outlay for the health sector.
- The Government of India aims to increase healthcare spending to 3 per cent of the Gross Domestic Product by 2022
- In February 2019, the Government of India established a new All India Institute of Medical Sciences at Manethi, District Rewari, Haryana at a cost of Rs 1,299 crore
- The Union Cabinet approved setting up of National Nutrition Mission (NNM) with a three-year budget of Rs 9,046 crore (US\$ 1.29 billion) to monitor, supervise, fix targets and guide the nutrition related interventions across ministries.
- In 2018, Government of India launched Pradhan Mantri Jan Arogya Yojana to provide health insurance worth Rs 500,000 to over 100 million families every year.

National Health Mission

The NHM envisages achievement of universal access to equitable, affordable and quality health care services that are accountable and responsive to people's needs. Its main programmatic components include Health System Strengthening, Reproductive-Maternal- Neonatal-Child and Adolescent Health and

Communicable and Non-Communicable Diseases with several schemes as follows:

- 1) **Reproductive, Maternal, Newborn, Child and Adolescent Health (RMNCH+A)** programme essentially looks to address the major causes of mortality among women and children as well as the delays in accessing and utilizing health care and services. It has introduced novel initiatives like the use of Score Card to track health performance, National Iron + Initiative to address the issue of anaemia across all age groups and the Comprehensive Screening and Early interventions for defects at birth, diseases, and deficiencies among children and adolescents.
- 2) **Rashtriya Bal Swasthya Karyakram (RBSK)** is an important initiative aiming at early identification and early intervention for children from birth to 18 years to cover 4 'D's viz. Defects at birth, Deficiencies, Diseases, Development delays including disability. Early detection and management diseases including deficiencies bring added value in preventing these conditions to progress to its more severe and debilitating form.
- 3) **The Rashtriya Kishor Swasthya Karyakram:** The key principle of this programme is adolescent participation and leadership, Equity and inclusion, Gender Equity and strategic partnerships with other sectors and stakeholders. The programme enables all adolescents in India to realize their full potential by making informed and responsible decisions related to their health and well-being and by accessing the services and support they need to do so.
- 4) The Government of India has launched **Janani Shishu Suraksha Karyakaram** to motivate those who still choose to deliver at their homes to opt for institutional deliveries. It is an initiative with a hope that states would come forward and ensure that benefits under JSSK would reach every needy pregnant woman coming to government institutional facility.

The National Health Mission encompasses its two Sub-Missions, The National Rural Health Mission and The National Urban Health Mission.

National Rural Health Mission (NRHM)

NRHM is an Indian health programme for improving healthcare delivery across rural India. NRHM was launched to address the infirmities and problems, prevailing across the primary healthcare system in the country. The mission aims to provide universal access to equitable, affordable, and quality healthcare that is accountable and at the same time responsive to the needs of the people. The following are the five planks of the mission:

- The Mission is expected to address the gaps in the provision of effective healthcare to rural population with a special focus on 18 states, which have weak public health indicators and/or weak infrastructure.
- The mission is a shift away from the vertical health and family welfare programmes to a new architecture of all-inclusive health development in which societies under different programmes will be merged and resources pooled at the district level.

- The mission aims at effective integration of health concerns, with a focus on major determinants of health and provision for flexible funding for states to utilise it in the areas they feel are important.
- The mission provides for appointment of Accredited Social Health Activists (ASHA) in each village and strengthening of the public health infrastructure, including outreach through mobile clinics. It emphasises involvement of the non-profit sector, especially in the underserved areas. It also aims at flexibility at the local level by providing for untied funds.
- The mission, in its supplementary strategies, aims at fostering public-private partnerships (PPPs) for improving equity concerns, reducing out of pocket expenses, introducing risk pooling mechanisms by social health insurance, and taking advantage of social health traditions.

National Urban Health Mission (NUHM)

- The NUHM will meet the health needs of the poor,
- particularly the slum dwellers through the availability of essential primary healthcare services by high-calibre health professionals,
- appropriate technology,
- public-private partnership and health insurance for urban poor.

The NUHM will ensure the following:

- Resources for addressing the health problems in urban areas, especially among urban poor.
- Need based city specific urban healthcare system to meet the diverse health needs of the urban poor and other vulnerable sections.

In India, approximately about 5.8 million people die because of diabetes, heart attack, cancer and other non-communicable diseases each year. Since the rate of deaths in the country because of communicable and non-communicable diseases is increasing at an alarming rate, the government has also introduced various programmes to aid people against these diseases.

National AIDS Control Organisation

National AIDS Control Organisation was set up so that every person living with HIV has access to quality care and is treated with dignity. By fostering close collaboration with NGOs, women's self-help groups, faith-based organizations, positive people's networks, and communities, NACO hopes to improve access and accountability of the services. It stands committed to building an enabling environment wherein those infected and affected by HIV play a central role in all responses to the epidemic – at state, district and grassroots level.

National TB Control Programme

Revised National TB Control Programme is a state-run tuberculosis control initiative of Government of India with a vision of achieving a TB free India. The programme provides, various free of cost, quality tuberculosis diagnosis and treatment services across the country through the government health system

National Leprosy Eradication Programme was initiated by the government for Early detection through active surveillance by the trained health workers and to provide Appropriate medical rehabilitation and leprosy ulcer care services.

Mission Indradhanush

The Government of India has launched Mission Indradhanush with the aim of improving coverage of immunization in the country. It aims to achieve at least 90 per cent immunization coverage by December 2018 which will cover unvaccinated and partially vaccinated children in rural and urban areas of India.

National Mental Health Programme

In order to address the huge burden of mental disorders and the shortage of qualified professionals in the field of mental health, Government of India has implemented National Mental Health Program to ensure the availability and accessibility of minimum mental healthcare for all in the foreseeable future.

Pulse Polio

Pulse Polio is an immunization campaign established by the government of India to eliminate polio in India by vaccinating all children under the age of five years against the polio virus.

The Pradhan Mantri Swasthya Suraksha Yojana (PMSSY)

PMSSY was announced with objectives of correcting regional imbalances in the availability of affordable/ reliable tertiary healthcare services and also to augment facilities for quality medical education in the country by setting up of various institutions like AIIMS and upgrading government medical college institutions. Rashtriya Arogya Nidhi

Since there are huge income disparities, therefore, the government has launched Rashtriya Arogya Nidhi which provides financial assistance to the patients that are below poverty line and are suffering from life-threatening diseases, to receive medical treatment at any government run super specialty hospital/ institution.

National Tobacco Control Programme

National Tobacco Control Programme was launched with the objective to bring about greater awareness about the harmful effects of tobacco use and about the Tobacco Control Laws and to facilitate the effective implementation of the Tobacco Control Laws.

Integrated Child Development Service(ICDS)

ICDS was launched to improve the nutrition and health status of children in the age group of 0-6 years, lay the foundation for proper psychological, physical and social development of the child, effective coordination and implementation of policy among the various departments and to enhance the capability of the mother to look after the normal health and nutrition needs through proper nutrition and health education.

Rashtriya Swasthya Bima Yojana

Rashtriya SwasthyaBima Yojana is a government-run health insurance programme for the Indian poor. It aims to provide health insurance coverage to the unrecognized sector workers belonging to the below poverty line and their family members shall be beneficiaries under this scheme.

Check Your Progress: 2

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) List 3 governmental healthcare schemes in India and give at least one feature of each.

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Modicare

Dubbed as the world's biggest experiment in universal health care, India launched its A\$2.2 billion universal health care plan in 2018 that gives 100 millions of its poorest people access to free health care to treat serious ailments. In practice, each will get a budget of 500,000 rupees, worth of free health care. It is a huge amount of money by Indian standards, particularly for those lower down the socio-economic scale. At the same time, 150,000 small doctor's surgeries and community health centres will be upgraded and receive better resources, as well as more government regulation. The scheme, known officially as Ayushman Bharat or the National Health Protection Mission but more popularly as Modicare, aims to cover 1300 illnesses, including cancer and heart disease. It is an enormous undertaking and a huge financial commitment, particularly for a country that, to date, has a poor record for health spending.

The Ayushman Bharat Yojana – National Health Protection Scheme, which has now been renamed as Pradhan Mantri Jan Arogya Yojana, aims to make secondary and tertiary healthcare completely cashless. The PM Jan Arogya Yojana beneficiaries get an e-card that can be used to avail services at an empanelled hospital, public or private, anywhere in the country with a coverage of 3 days of pre-hospitalisation and 15 days of post-hospitalisation expenses.

Check Your Progress: 3

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) What has been deemed as the world's biggest experiment in UHC? Give at least two of its key elements.

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9.3 NON-GOVERNMENTAL INITIATIVES

Recent years have seen a growing capacity of nongovernmental organizations to develop patterns of cooperation among themselves locally, nationally, and internationally, for consultation and exchange of information, or for joint action. In the area of Primary Health Care and Development NGOs can play pivotal roles in the two major developmental approaches as Integrated Human Development and Community Participation. Non-governmental organizations (NGOs) are called by various names across the world, such as third sector organizations, non-profit organizations, voluntary organizations, charitable organizations, community-based organizations and so on with slightly modified scope and coverage.

9.3.1 Non-Governmental Organizations in Healthcare System

Non-governmental organizations typically possess a very distinctive feature as private institutions serving public purposes (UN 2003). Though their nature and focus of activities has changed over time, they have gained prominence in a wide spectrum of activities cutting across economic, social, cultural and scientific domains. Nowadays, non-governmental sector is recognized as a major social and economic force in almost all countries.

The importance of evolving non-governmental sector is well explained in the literature of political economics as government and market failure theories. Sociological studies emphasize on them as a most important institution improving social integration and being an integral part of democratic society. The World Health Organization has acknowledged NGOs in terms of increasing recognition to complement government programs and creating an effective people's voice in respect of health service requirements and expectations. With the adoption of a decentralized framework to improve health sector performance, many countries across the globe have opened up the opportunity for participation of NGOs in providing health services.

However, the tasks of NGOs in healthcare sector are related to specifics of a particular country, namely the extent of civilization development, institutional framework, culture and tradition, resources and needs etc. In high income countries, NGOs are typically more specialized and largely involved in clinical research, health advocacy and lobbying. In countries with low and middle income, NGOs activities are focused on service delivery, raising awareness and prevention campaigns.

9.3.2 Global and Local Perspective

Since the mid-seventies, the non-governmental sector has experienced a phenomenal growth in both developed and developing countries. This is perhaps caused by the policy shift towards decentralized agenda of both national and international development agencies. Decentralization has opened up important avenues for NGOs to participate in social sector programs. Governments in the

developing world especially found the non-profit sector as increasingly prominent, innovative and grassroots driven with the desire and capacity to pursue participatory and people-centred forms of development and to fill up the gaps left by governments in meeting the needs of vulnerable citizens. NGOs working directly with local communities can play an important role in extending project uptake and reach and can facilitate greater awareness of diverse stakeholder views.

In India, NGO-run hospitals are heterogeneous and vary in terms of ownership, financing and costs. In recent past, in about ten health-oriented projects of the Ministry of Health and Family Welfare, Government of India, NGOs have actively taken part as health service providers to the financing agent (fund management) based on their level of capacity. Besides, some NGOs (especially the national counterparts of international NGOs and faith-based organizations) might have their own health financing schemes. Those that are present in rural areas and part of development programs, function for the community by providing primary and secondary services. For example, SEWA-Rural in Gujarat, was assigned the responsibility of providing community-based health services in a district covering 40 villages with a population size 35,000. Alongside health care facilities, NGOs are also involved in several preventive care activities, perhaps more than curative care.

For over 50 years, USAID's global health programs have saved lives, protected people most vulnerable to disease, and promoted the stability of communities and nations, while advancing American security and prosperity. America is safer and stronger when people can live healthy and productive lives and when nations around the world are self-reliant and resilient.

Check Your Progress: 4

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) List down any two significant features of non-governmental initiatives in the health care system.

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9.3.3 Non-Governmental Global Healthcare Initiatives

USAID's global health efforts grounded in investments in health systems strengthening and breakthrough innovation, are focused around three strategic priorities: Preventing child and maternal deaths; controlling the **HIV/AIDS epidemic**; and **combating infectious diseases**. Learn more about our different health areas below:

- **Preventing Child and Maternal Deaths**

Through efforts in family planning, maternal and child health, malaria, and nutrition, USAID works to prevent child and maternal deaths by helping women and children access essential, and often life-saving, health services.

- **Controlling the HIV/AIDS Epidemic**

Since 1986, USAID's HIV/AIDS program has been on the forefront of the global AIDS crisis, providing strategic direction, leadership, and expertise to help control one of the world's most serious public health challenges.

- **Combating Infectious Diseases**

In an increasingly interconnected society, a health threat anywhere is a threat everywhere. For decades, USAID has been a leader in the fight against infectious diseases, including malaria, through the U.S. President's Malaria Initiative; HIV/AIDS, through PEPFAR; tuberculosis; neglected tropical diseases; pandemic influenza, and other global health security challenges.

- **Health Systems**

Strong and resilient health systems are necessary for achieving sustained positive health outcomes. By linking health issues through integrated approaches, USAID contributes to sustainable and cost-effective programmes that leave a lasting impact on country health systems.

- **Innovation and Impact**

USAID supports breakthrough innovations, applies market-based approaches, and advances efforts in digital health, private sector engagement, and human-centred design in order to maximize the impact of its global health work.

Check Your Progress: 5

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) How do USAID's investments and efforts in India contribute to health care in India?

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9.3.4 Non-Governmental Healthcare Organizations in India

In India, it was the 1970s which saw rapid growth in the formation of formally registered NGOs and the process continues to this day. Most NGOs have created their respective thematic, social group and geographical priorities such as poverty alleviation, community health, education, housing, human rights, child rights, women's rights, natural resource management, water and sanitation; and to these ends they put to practice a wide range of strategies and approaches. Primarily, their focus has been on the search for alternatives to development thinking and practice; achieved through participatory research, community capacity building and creation of demonstrable models.

Voluntary Health Association of India (VHAI)

Voluntary Health Association of India (VHAI) is a non-profit, registered society

formed in the year 1970. It is a federation of 27 State Voluntary Health Associations, linking together more than 4500 health and development institutions across the country. They are one of the largest health and development networks in the world. VHAI advocates people-centered policies for dynamic health planning and programme management in India. VHAI promotes health issue of human right and development. The beneficiaries of VHAI's programme include health professionals, researchers, social activists, government functionaries and media personnel. FSSAI and Ministry of Health & Family Welfare, GOI has given an appreciation award to VHAI for its remarkable work on Eat Right Movement and Swasth Bharat Yatra.

HelpAge India

HelpAge is one of the leading charitable organizations in India which work for the well-being of the elderly. It was established in 1978, and has been providing services for over 4 decades now. They provide free health services for the unfortunate elderly with cataract surgeries and intensive care to end-stage cancer patients. HelpAge India has received several awards for his commendable contribution to society including the Chairman's Challenge Award, Times Social Impact Award and NGO Leadership and Excellence Award among several others.

CRY-Child Rights and You

CRY was started by Rippan Kapur in 1979 aiming to work towards several causes for children, one of them being malnutrition. It introduced kitchen gardens in Anganwadis in Chhattisgarh to provide fresh and healthy food to children. This noble initiative of CRY has helped to see a decrease in the number of malnourished children by about 9-10 per cent. Over the years CRY has won numerous awards that recognize their efforts in giving India's children a happy and healthy childhood. It holds The Institute of Chartered Accountants of India Corporate Social Responsibility Award for Best CSR Project in Health, Lakshya Award, Karve Institute of Social Service for 50 years of excellence etc.

Rural Healthcare Foundation (RHCF)

RHCF strives to provide low cost primary healthcare to the lowest strata of the socio-economic pyramid and is doing well in achieving its goals. RHCF is currently a network of 10 clinics, spread across various districts of West Bengal and has since inception served over 10 lakh patients through its network. Rural Health Care Foundation been given a Special Consultative Status at the United Nations and recognition by the Harvard Business School.

Udaan Welfare Foundation

Udaan Welfare Foundation was found in 2008 with an objective to work towards quality healthcare and nutrition through several programs. Udaan carried out a dental health check in the Harikishan English Public School and taught 250 students the importance of the correct brushing techniques and oral care. They have also regularly provided medicines and monthly ration supply for Anugraha Children's Home and Anugraha Vidya Mandir, Ambarnath since 2008. It has received awards and recognition including, the Galaxy Recognition Award, Samaj Shakti Award, Global CSR Excellence and Leadership Award etc.

Uday Foundation

Established in 2006 by Rahul and Tulika Verma, Uday Foundation is a Delhi based NGO, supporting children suffering from congenital disorders and other syndromes. The Foundation also researches new technologies in the healthcare sector to serve wholesome food and raising donations. It is a very renowned foundation, featuring in The times of India, NDTV etc.

**Health Care:
Governmental and
Non-Governmental
Initiatives**

India Health Action Trust (IHAT)

India Health Action Trust was constituted by the registered Trust Deed in 2003 dedicated to the enhanced health and wellbeing of individuals and communities in India, irrespective of caste, creed or religion. IHAT originally focused on providing comprehensive technical assistance and training in programme planning and management to the states of Karnataka and Rajasthan. Over the years, the trust has supported the State AIDS Control Societies (SACS) in Maharashtra, Bihar, Rajasthan, Andhra Pradesh, Tamil Nadu and Goa. IHAT Senior Technical Advisor awarded with Honoris Causa Recognising the exceptional contribution towards the society, by the University of Manitoba on their 31st convocation.

Vatsalya NGO

Vatsalya was set-up as a Resource Centre on Health in the year 1995 by medical professionals with the objective to make consistent efforts towards ensuring quality health services especially to the poor and marginalized community of rural areas. Vatsalya has been contributing to Uttar Pradesh for last 17 years towards enhancing capacity of individuals and communities to understand comprehensive framework of health in terms of science and art. They received awards from HSBC Water Programme, for water conservation, and were also chosen as finalists in the dasra awards in the sanitation category.

Check Your Progress: 6

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) List at least two NGOs and how are they contributing alleviate healthcare in the country.

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9.4 LET US SUM UP

Within the wide framework of nested health missions, policies, programmes, acts and statues lie the overarching objective of providing good health and well being to all. While the role of government needs to be enhanced along with more revenue allocation for successful results in health sector, contribution to health of a population derives from systems outside the formal health care system, and this potential of intersectoral contributions to the health of communities is being increasingly recognized worldwide. Thus, the role of government in influencing

population health is not limited within the health sector but also by various sectors outside the health systems.

While a lot has been achieved in the years gone by – the launch of Expanded Program of Immunisation in 1974, Primary Health Care enunciated at Alma Ata in 1978, eradication of Smallpox in 1979, launch of polio eradication in 1988, FCTC ratification in 2004 and COTPA Act of 2005, to name a few, the future of a healthy India lies in mainstreaming the public health agenda in the framework of sustainable development. The ultimate goal of great nation would be one where the rural and urban divide has reduced to a thin line, with adequate access to clean energy and safe water, where the best of health care is available to all, where the governance is responsive, transparent and corruption free.

In this changing world, with unique challenges that threaten the health and well-being of the population, it is imperative that the government and community collectively rise to the occasion and face these challenges simultaneously, inclusively and sustainably. Social determinants of health and economic issues must be dealt with a consensus on ethical principles – universalism, justice, dignity, security and human rights. This approach will be of valuable service to humanity in realizing the dream of Right to Health.

NGOs are contributing at all stages of the research cycle, fostering the relevance and effectiveness of the research, priority setting, and knowledge translation to action. They have a key role in stewardship (promoting and advocating for relevant global health research), resource mobilization for research, the generation, utilization and management of knowledge, and capacity development. Yet, typically, the involvement of NGOs in research is downstream from knowledge production and it usually takes the form of a partnership with universities or dedicated research agencies. There is a need to more effectively include NGOs in all aspects of healthcare in order to maximize the potential impact. NGOs, moreover, can and should play an instrumental role in coalitions for global health. With a renewed sense of purpose and a common goal, NGOs and their partners intend to make strong and lasting inroads into reducing the disease burden of the world's most affected populations through effective research and action.

To get to the envisioned future of Sustainable Development Goals' 'Health For All', health care providers, governments and other payers, disruptive entrants, and even consumers should begin working in cohesion driving change across the ecosystem to lay a solid foundation for the future.

9.5 KEY WORDS

Healthcare: Health care is the maintenance or improvement of health via prevention, diagnosis, treatment, recovery or cure of disease, illness, injury or physical and mental impairments in people.

Obamacare: An informal term for the Affordable Care Act, a federal law intended to improve access to health insurance for US citizens.

Modicare: The Ayushman Bharat Yojana - National Health Protection Scheme, which has now been renamed as Pradhan Mantri Jan Arogya Yojana is India's

A\$2.2 billion universal health care plan that aims to give 100 millions of its poorest people access to free health care to treat serious ailments.

National Health Mission: The National Rural Health Mission (NRHM), now under National Health Mission is an initiative undertaken by the government of India to address the health needs of under-served rural areas.

Non-Profit Organisation: A non-profit organization that operates independently of any government, typically one whose purpose is to address a social or political issue.

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9.7 CHECK YOUR PROGRESS: POSSIBLE ANSWERS

Check Your Progress: 1

1. Refer to Section 9.2.1

Check Your Progress: 2

1. Refer to Section 9.2.4

Check Your Progress: 3

1. Refer to Section 9.2.4 (pg 148)

Check Your Progress: 4

1. Refer to Section 9.3

Check Your Progress: 5

1. Refer to Section 9.3.3

Check Your Progress: 6

1. Refer to Section 9.3.4

UNIT 10 HEALTH INFRASTRUCTURE AND DELIVERY SYSTEM

Structure

- 10.0 Introduction
- 10.1 Learning Outcomes
- 10.2 Health Infrastructure in India
- 10.3 Healthcare Infrastructure in Public Sector
 - 10.3.1 India's Medical Infrastructure at Glance
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 - 10.5.1.1 Union Ministry of Health and Family Welfare
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 - 10.6.2 Primary Health Care
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- 10.8 Key Words
- 10.9 Further Readings
- 10.10 Check Your Progress: Possible Answers

10.0 INTRODUCTION

Health is often viewed as an individual attribute, to be enjoyed, attended to and mended at a personal level. This perspective imposes the responsibility of ill health on the individual and also heaps the physical, financial and emotional burdens of obtaining healthcare on that person. Health status of a population does not remain static. India's health system has many mixes: state and central health services; public and private and voluntary sectors; allopathic and non-allopathic doctors, qualified and unqualified practitioners and multiple levels of health delivery systems. The delivery of healthcare has two important elements

i.e., health financing and manpower through health delivery systems. The health systems need to be financed adequately to develop and maintain the required infrastructure, employ an adequate number of skilled health professionals of different categories. The health workforce includes the doctors and other allied health professionals. The shortages of health care forces result in the unsatisfactory of health care to the community.

10.1 LEARNING OUTCOMES

After reading this unit, you will be able to:

- explain Health Infrastructure;
- model of Health Delivery System; and
- critical evaluation of health delivery system.

10.2 HEALTH INFRASTRUCTURE IN INDIA

Health is a fundamental human right and a global social goal. It is pertinent for the realization of basic human needs and for a better quality of life. Health is also considered a causative factor that affects country's aggregate level of economic development. Since development is a consequence of good health, even the poorest developing countries should make it a priority to invest in the health sector. Unfortunately, health has been poorly invested in by countries with low human development and the health sector still remains largely untapped and continues to suffer neglect.

Health infrastructure generally mean to the outbreak of epidemics, improvement of environmental sanitation and provision for some hospitals, health centers and dispensaries scattered in various parts of the country to provide medical relief and promote health conditions. The pattern of health infrastructure promotes future health development programmes in the country. In this context it is also stressed that the health services should be provided for the vast rural population of the country and the health services should be placed as close to the people as possible in order to ensure the maximum benefit to the people of the communities.

The rank of India in the Human Development Index Report 2018 (130 out of 189 countries) issued by UNDP depicts the level of ignorance of the health sector in a country like India. Recently due attention is paid on the various facets of health problems by central and Provincial Health Departments.

10.3 HEALTHCARE INFRASTRUCTURE IN PUBLIC SECTOR

India is one of the fastest growing economy of the world. The very essential components of Primary Health Care is promotion of food supply, proper nutrition, safe water and basic sanitation and provision for quality health information. It is observed that health care services, provision of essential medicines, scarcity of Doctors and other paramedical staffs are bottlenecks in the primary health care scenario.

Parameter	First Plan (1951-56)	(2015)
Primary Health Centre	725	25,650
Sub Centres	84,376 (1985)	156,231
Community Health Centres	761(1985)	5,624
Total Beds	125,000	914,543
Medical Colleges	42	476 (2017)
Dental Colleges	7	313
Allopathic Doctors	65000	1,041,395 (2017)
Nurses	18500	21,51,850
ANMs	1,27,800	8,92,829
Health Visitor	578	56,644
Health Workers(F)	133194	212,185
Health Workers (M)	-	62,881
Village Health Guides	-	323,000
AYUSH Doctors	NA	773,668

10.3.1 India's Medical Infrastructure at Glance

Number of Community Health Centres (CHC) functioning in government buildings has increased during the period 2005-2017. The percentage of CHCs in Government buildings has increased from 91.6 per cent in 2005 to 96.7 per cent in 2017.

In addition to 4156 Specialists, 14350 General Duty Medical Officers (GDMOs) are also available at CHCs on 31st March, 2017. There was huge shortfall of surgeons (86.5 per cent), obstetricians and gynecologists (74.1 per cent), physicians (84.6 per cent) and pediatricians (81 per cent). Overall, there was a shortfall of 81.6 per cent specialists at the CHCs vis-à-vis the requirement for existing CHCs.

Financial Position

- Total public expenditure on health 5.2 per cent of GDP
- Public Investment 0.9 per cent of GDP
- Budget allocation for health 1.3 per cent of central budget
- Government Expenditure 25 per cent
- Out of pocket expenditure 75 per cent
- Central government contribution to state 15 per cent
- State Budgetary allocations reduced from 7 per cent to 5.5 per cent

- India's health budget has gone up nearly 4000 crore to Rs. 21113.33 crore (\$4.35 billion)

Check Your Progress: 1

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) Mention the Health Infrastructure in India.

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10.4 DELIVERY OF PUBLIC HEALTH CARE IN INDIA

Healthcare is one of the India's largest service sector. Under the Indian Constitution, health is a state subject. Each state therefore has its own healthcare delivery system in which both public and private (for profit as well as non-profit actors operate). While states are responsible for the functioning of their respective healthcare systems, certain responsibilities also fall on the federal (central) government, namely aspects of policy-making, planning, guiding, assisting, evaluating and coordinating the work of various provincial health authorities and providing funding to implement national programmes.

10.4.1 Model of Health Care Delivery System in India

India is a union of 28 states and 8 union territories. States are largely independent in matters relating to the delivery of health care to the people. Each state has developed its own system of health care delivery, independent of central government.

The organization at the national level consists of the Union Ministry of Health and Family Welfare. In each state, the organization is under the State Department of Health and Family Welfare that is headed by a State Minister and with a Secretariat under the charge of the Secretary/Commissioner (Health and Family Welfare).

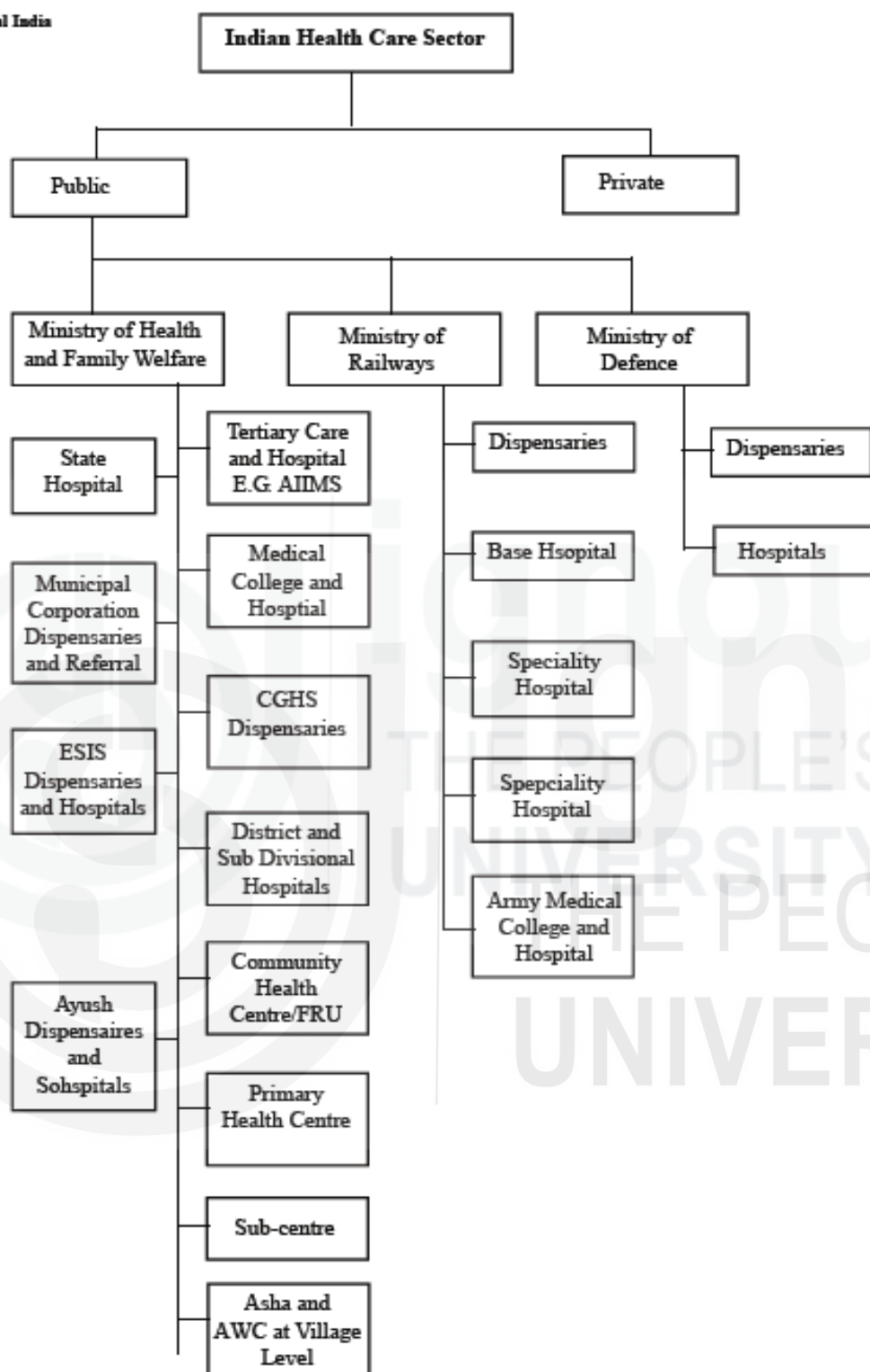


Fig. 10.1 Health Delivery System in India

Check Your Progress: 2

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) What is Public Health Care Delivery system?

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- 2) Present the model of Health Care Delivery in India.

10.5 ORGANS OF HEALTH DELIVERY SYSTEMS

The central government responsibility consists mainly of policy making, planning, guiding, assisting, evaluating and coordinating the work of the State Health Ministries. The health system in India has three main links:

- 1) Central
- 2) State
- 3) Local and Peripheral

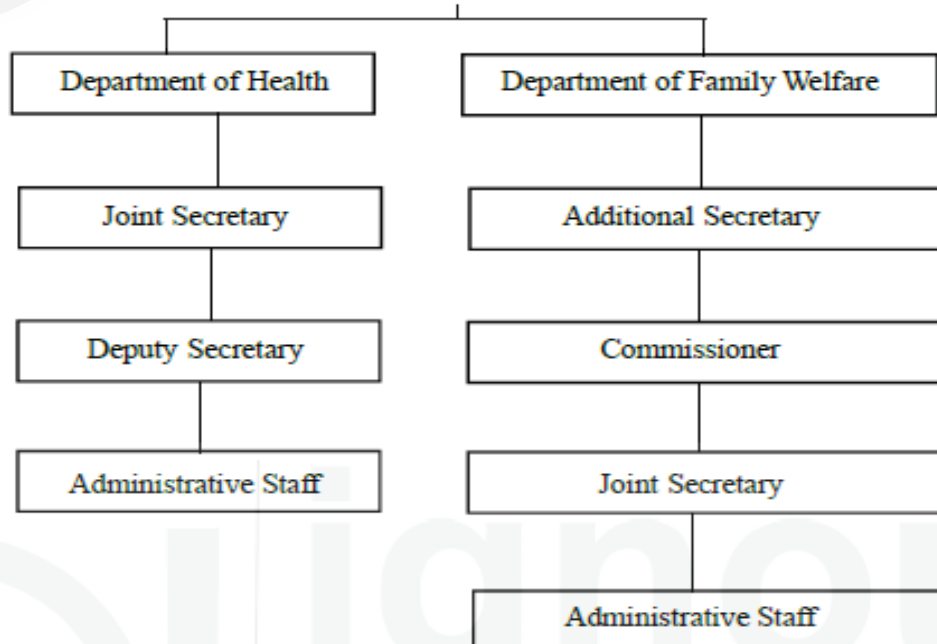
10.5.1 Health Delivery at Central Level

The Official organs of the health system at the national level consist of:

- Ministry of Health and Family Welfare
- The Directorate General of Health Services
- The Central Council of Health and Family Welfare

10.5.1.1 Union Ministry of Health and Family Welfare

Hierarchical Structure of Health Care Administration
Cabinet Minister



Union List

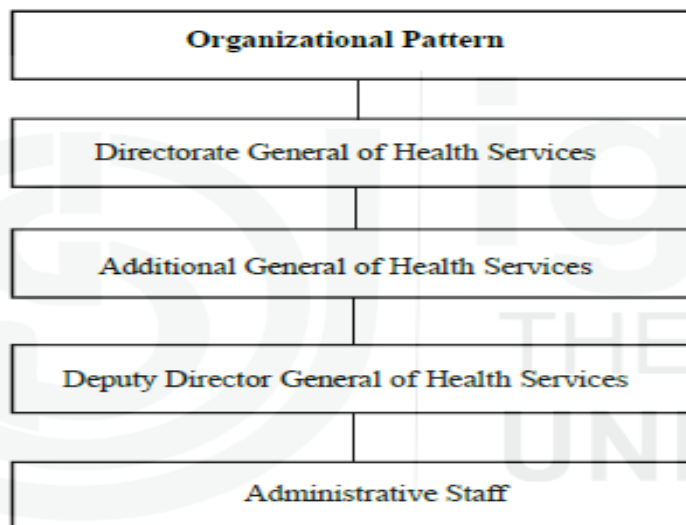
- International health relations and administration
- Administration of Central Institutes such as All India Institute of Hygiene and Public Health, Kolkata
- Promotion of research through research centres
- Regulation and development of medical, pharmaceutical, dental and nursing profession
- Establishment and maintenance of drug standards
- Census and collection and publication of other statistical data
- Immigration and Emigration
- Regulation of labour in the working of mines and oil fields
- Coordination with states and with other ministries for promotion of health.

Concurrent List

The functions listed under the concurrent list are the responsibility of both the union and state governments:

- Prevention and extension of communicable diseases
- Prevention of adulteration of food stuffs
- Control of drugs and poison
- Vital statistics
- Labour Welfare
- Ports
- Economic and social Planning
- Population control and Family Planning
- Preparation of health education material for creating health awareness through Central Health Education Bureau
- Collection, compilation analysis, evaluation and dissemination of information through the Central Bureau of Health Intelligence
- National Medical Library.

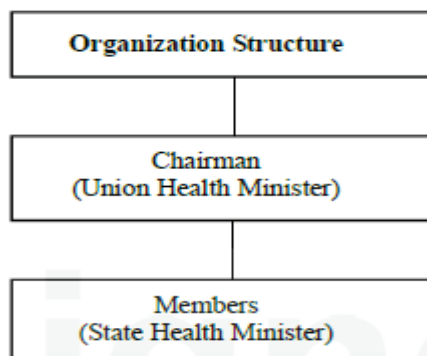
10.5.1.2 Directorate General of Health Services



Functions

- International health relations and quarantine of all major ports in country and International airport
- Control of drug standards
- Maintain medical store depots
- Administration of post-graduate training programmes
- Administration of certain medical colleges in India
- Conducting medical research through Indian Council of Medical Research
- Central Government Health Scheme
- Implementation of National Health programmes
- Preparation of health education for creating health awareness through Central Health Education Bureau
- Collection, compilation, analysis, evaluation and dissemination of information through the Central Bureau of Health Intelligence
- National Medical Library.

10.5.1.3 Central Council of Health



Functions

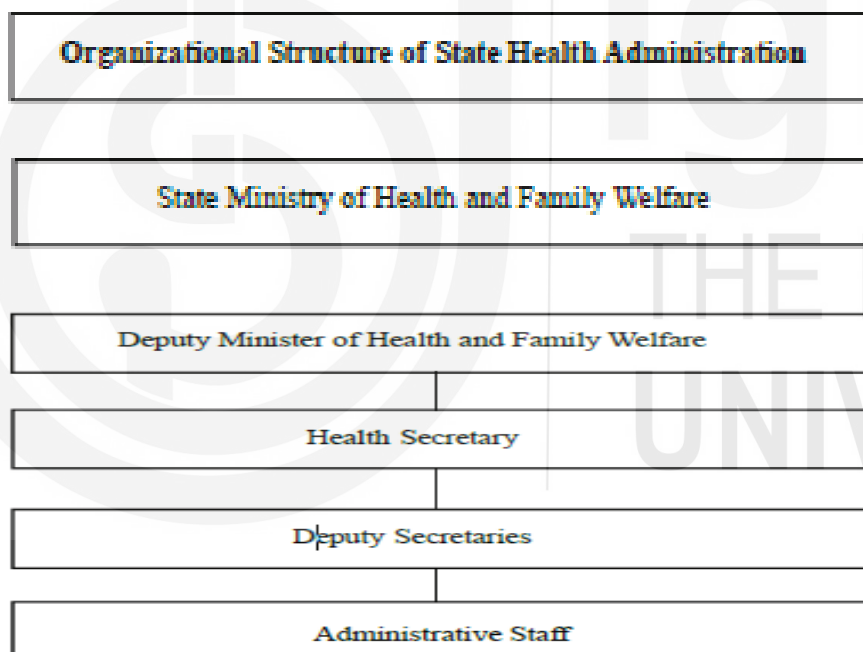
- To consider the recommended broad outlets of policy regard to matters concerning health like environment hygiene, nutrition and health education
- To make proposals for legislation relating to medical and public health matters
- To make recommendations to the Central Government regarding distribution of Grants-in-aid.

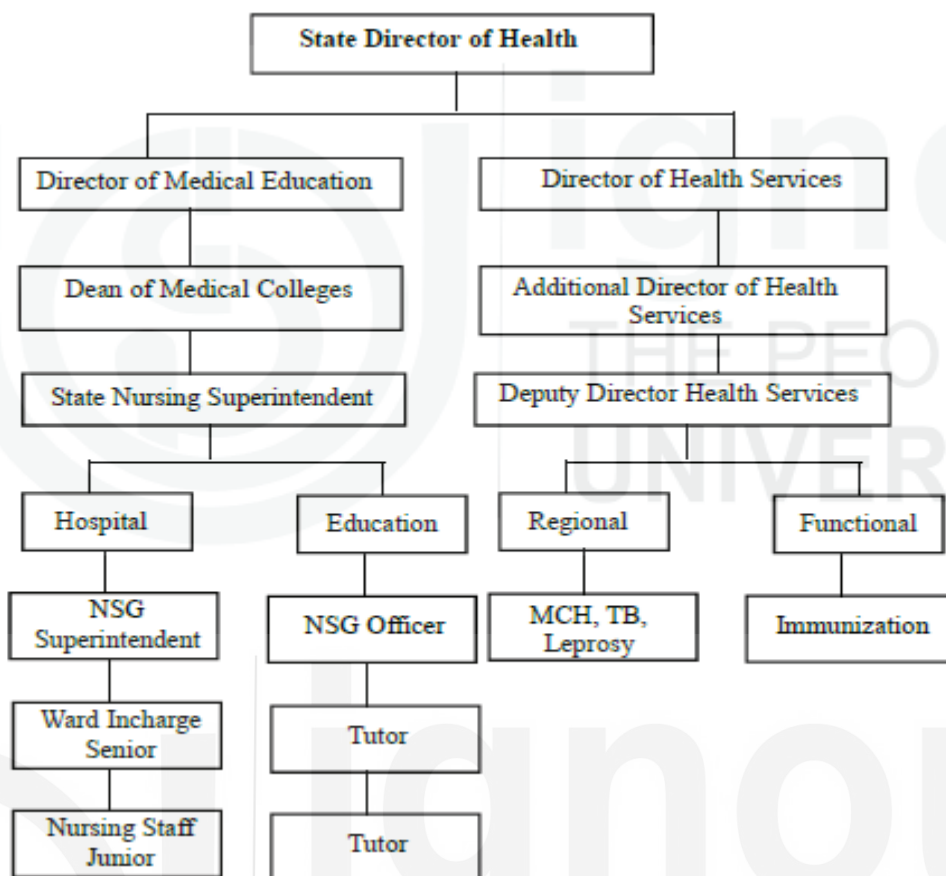
10.5.2 State Level Health Care

The health subjects are divided into three groups: federal, concurrent and state. The state list is the responsibility of the state, including provision of medical care, preventive health services and pilgrimage within the state.

State Health Administration

At present there are 28 states in India, each state having its own health administration.





Functions of State Health Director

- Studies in depth and health problem and needs in the state and plans scheme to solve them
- Providing curative and preventive services
- Provision for control of milk and food sanitation
- Prevention of any outbreak of communicable diseases
- Promotion of health education
- Promotion of health programmes such as school health, family planning, occupational health
- Supervision of PHC
- Establishing training outcomes for health personnel
- Co-ordination of all health services with other minister of state such as minister of education, central health minister and voluntary agency.

10.5.3 District Level Health Care

In each district in India, there are six types of administrative areas.

- 1) Sub-division
- 2) Tehsils (Taluku)
- 3) Community Development Blocks

- 4) Municipalities and Corporations
- 5) Villages and
- 6) Panchayats.

The Functions of Municipal Board:

- Construction and maintenance of roads
- Sanitation and drainage
- Street Lighting
- Water supply
- Maintenance of Hospital and Dispensaries
- Education
- Registration of births and deaths etc.
- The corporations are headed by Mayor, elected by councillors who are elected from different wards of the city.

Village Level: Panchayati Raj

The panchayat raj is a three-tier structure of rural local self-government linking the villages to the district includes:

- Panchayat (at the village level)
- Panchayat samiti (at the block level)
- Zila Parishad (at the district level)

10.5.4 Health Care System in Rural Areas

- 1) At village level
- 2) At the centre level
- 3) At Primary Health Centre level
- 4) At Community Health Centre level.

At the village level, the elementary services are rendered by:

- 1) **Village Health Guides:** Village health guides is a person with an aptitude for social service and is not full time government functionary. Village health guide scheme was introduced on 2nd October, 1977. He provides the treatment for common minor ailments; first aid during accidents and emergency; MCH care; family planning; health education.
- 2) **Local Dais:** Most deliveries in rural areas used to handle by *untrained dais*. The training for dais is given for 30 working days. Each *dai* is paid stipend of Rs. 300 during the period of training. During her training each *dai* is required to conduct at least 2 deliveries under the supervision and guidance of health worker (female), Auxiliary Nurse and Midwife, health assistant (female). *Dai* perform the function of MCH care; family planning; immunization; education about health, referral services; safe water and basic sanitation and nutrition.

- 3) **Aganwadi workers:** Under the Integrated Child Development Scheme (ICDS) there is an aganwadi worker for a population of 1000. There are about 100 such workers in each ICDS project. The aganwadi worker is selected from the community and she under goes training in various aspects of health, nutrition and child development for four months. She is a part time worker and paid an honorarium. She performs Maternal and Child Health (MCH) Care; family planning; immunization, education about health, referral services, safe water and basic sanitation, supplementary nutrition and non-formal education of children.
- 4) **Accredited Social Health Activist (ASHA):** One of the key components of National Rural Health Mission is to provide every village in the country with a trained female community health activist–ASHA, selected from the village itself and accountable to it. The ASHA is trained to work as interface between the community and the public health system. One ASHA on the population of 1000. ASHA must be primarily a woman from the resident of the village itself. ASHA will take steps to create awareness and provide information to the community on determinants of health such as nutrition, basic sanitation and hygiene practice, healthy living and working conditions, information on existing health services and the need for timely utilization of health and family welfare services. She will counsel women on birth preparedness, importance of safe delivery, breast-feeding and complementary feeding, immunization, contraception and prevention of common infections including Reproductive Tract Infection, Sexually Transmitted Infections and care of the young child. ASHA will mobilize the community and facilitate them in assessing health and health related services available at the village/ sub-centre/primary health centres, such as immunization, ante-natal checkup (ANC), Post-natal check-up (PNC), ICD, sanitation and other services being provided by the government. She will arrange escort/accompany pregnant women and children requiring treatment/ admission to the nearest pre-identified health facility. ASHA will provide primary medical care or minor ailments such as diarrhea, fever, and first aid for minor injuries. She will also inform about the births and deaths in her and any unusual health problems/disease outbreaks in the community to the sub-centre/primary health centre. She will also promote construction of household toilets under total sanitation campaigns.

Check Your Progress: 3

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

- 1) What are the Organs of Health Delivery Systems?

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- 2) Mention the hierarchy of Central Council of Health Administration.

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3) Give the Model of State Health Administration.

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10.6 CRITICAL EVALUATION OF FUNCTIONING OF HEALTH DELIVERY

There is still a short fall of 16 per cent of PHCs and 58 per cent of CHCs; Non availability of staff; Weak referral system; Recurring funding shortfalls; and Lack of accountability for quality of care.

10.6.1 Problems in Public Health Care

Very low use of massive Public health infrastructure; Poor availability and access; and Unsatisfactory work of public health units. Only 20 per cent of OPD and 45 of inpatient care obtained from government health infrastructure while rest is from private sector.

10.6.2 Primary Health Care (PHC)

It is important to emphasize that PHC is not just a descriptive term denoting the first level of health care. It is comprehensive view of health that emerges from the most critical conceptual advances of twentieth-century public health. Unlike the techno-centric approaches that derive from the bio-medical sciences– such as linear campaigns against small pox and malaria– the PHC approach confronts complex socio-economic, political, and technological relationships. The emphasis of PHC is on:

- Equity in health care;
- Need-based, socially acceptable services with full participation of people;
- State responsibility for incorporating PHC into national development plans through inter-sectoral strategies;
- Affordable technologies ensuring self-sufficiency and effective basic health care with the support of secondary and tertiary care; and
- Collective, not individual efforts.

The repeated failures of technocentric approaches and researches that emphasized the link between socio-economic and demographic factors and health transitions led to the 1978 Alma-Ata Declaration. It stated that mere formal government support would not be sufficient to achieve the desired level of health and that PHC would require a reorientation of national strategies for health planning. It would imply a transfer of health resources to the under-served. The health budget might have to be increased until the total population received essential care and institutions supporting PHC were made effective.

Staff composition for each Primary Health Care (PHC) which includes Family Medical Practitioner (FMP) unit to include four doctors, one PHN, two nurse midwives, eight ANMs (females), four Multi-Purpose Workers (MPWs) (males), one pharmacist, one clerk/assistant, one office assistant, one lab technician, one driver, one sweeper– this adds up to salaries and benefits/capitation of Rs. 6 million (salary structures across states may be different and hence this could vary). Doctors and nurses may either be salaried or contracted. The curative care component should work as a family medical practice with families (500-2000, depending on density) being assigned to each provider. There should be 10 beds per primary health centre. Average rural unit to cover 20,000 population (in the range 10-30 thousand depending on density); average urban unit to cover 50,000 population (in the range 30-70 thousand population depending on density). In rural areas for every 5 PHCs there would be one 50 bedded hospital and this would cost Rs. 400,000 per bed per annum or Rs. 20 million per such hospital. As per this ratio we would need 7500 rural hospitals and this would translate into Rs. 150 billion for the country as a whole. In urban areas for each 10 PHCs one 200 bedded hospital would be needed and this would cost Rs. 500,000 per bed per year or Rs. 100 million per hospital. As per this ratio 700 such hospitals would be needed and this would translate into Rs. 70 billion for the country as a whole.

It is thus evident that the neglect of the public health system is an issue larger than government policy making. The latter is the function of the overall political economy. Under capitalism only a well-developed welfare state can meet the basic needs of its population. Given the backwardness of India the demand of public resources for the productive sectors of the economy (which directly benefit capital accumulation) is more urgent (from the business perspective) than the social sectors, hence the latter get only a residual attention by the state. The policy route to comprehensive and universal healthcare has failed miserably. It is now time to change gears towards a rights-based approach. The opportunity exists in the form of constitutional provisions and discourse, international laws to which India is a party, and the potential of mobilizing civil society and creating a socio-political consensus on right to healthcare. There are a lot of small efforts towards this end all over the country, for instance the People's Health Movement or the Jan Swasthya Abhiyan (JSA). Synergies have to be created for these efforts to multiply so that people of India can enjoy right to healthcare. On the government front the UPA government of Manmohan Singh promises to restructure the public health system via one of its flagship programs, the National Rural Health Mission (NRHM).

The preamble of the NRHM document states, “Recognizing the importance of Health in the process of economic and *social development and improving the quality of life of our citizens, the Government of India has resolved to launch the National Rural Health Mission to carry out necessary architectural correction in the basic health care delivery system.... The Goal of the Mission is to improve the availability of and access to quality health care by people, especially for those residing in rural areas, the poor, women and children*” (Ministry of Health and Family Welfare: 2005). This goal will be achieved by strengthening the three levels of rural healthcare – the subcenters, PHC and CHC. At the village/hamlet level a health worker called ASHA (Accredited Social Health Activist) will be

appointed who will be the link worker for rest of the public health system in rural areas. Additional resource allocation and up gradation of the facilities at each level has been planned under the Mission.

To conclude, the NRHM should be used as an opportunity to work out a new health financing strategy, which devolves financial resources to local governments and uses a social audit framework to monitor its implementation. Only this will lead to structural changes and improvements in the health of the Indian people. But the government will not do this until healthcare becomes a political agenda and drives elections. Thus, the civil society has to exert pressure from below. The initiatives of the Jan Swasthya Abhiyan, which has set in motion a campaign on Right to Healthcare, is one such move in this direction. The JSA has formulated a Peoples Health Charter, has mobilized groups in 18 states to support the campaign on right to healthcare, has collaborated with the National Human Rights Commission to conduct public hearings on denial of healthcare across the length and breadth of the country and is now actively engaging with the NRHM initiative to not only monitor its progress through community involvement but also use it as an opportunity to restructure health and healthcare in India.

10.6.3 Secondary and Tertiary Care/Teaching Hospitals

Secondary Health Care: Secondary health care is at district levels – where most of the conditions are treated there itself. Only the complex ones are referred. Secondary healthcare includes specialists like cardiologists, dermatologists, urologists, and others. Individuals reach bent secondary medical aid providers through the referral of primary healthcare professionals. In a few countries, the individuals cannot consult the specialists without the referral of the medical man at the first care level. The secondary healthcare providers act as a liaison between the patient and therefore the advanced medical aid. The systems that come under this category are referred to as the district health system and county health system.

District health system mainly focuses on child health and maternity care. Healthcare centres receive referrals from various primary healthcare centres and remain open for 24 hours a day. District hospitals include emergency services, neonatal care, comprehensive emergency obstetric.

Tertiary Healthcare

Tertiary healthcare is provided in hospitals with government medical colleges having a specialty and super specialty departments. This ranks on top within the hierarchy. Research centres also fall under tertiary healthcare. This is the care that comes into the image as a referral to patients by the first and healthcare providers. The individuals may require advanced medical procedures like major surgeries, transplants, replacements, and long-term medical aid management for diseases like cancer, or neurological disorders.

Specialised conductive medical aid is the highest sort of healthcare practice and performs all the main medical procedures. Advanced diagnostic centres, specialised medical care units, and modern medical facilities are the key

features in tertiary medical aid. The practices that provide tertiary medical aid might be a part of the government or a mixture of both public and personal sectors. The healthcare system in India is an example of the latter.

The health providers who are a part of these three levels of healthcare categories of India, together provide medical services like health issue finding, evaluating, providing treatment, or pertaining to the subsequent level of care supported the health needs. The ever-increasing demand for health needs is pushing the borders of healthcare, forcing it to tread the new paths. Managing a sector with such huge potential might be driven within the success path with the assistance of healthcare consulting firms in India.

Check Your Progress: 4

Note: 1) Use the space provided below for your answers.

2) Compare your answers with those given at the end of the unit.

1) Critically evaluate functioning of Health Care delivery system?

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2) Describe secondary health care system?

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10.7 LET US SUM UP

Healthcare is one of the India's largest service sectors. Under the Indian Constitution, health is a state subject. Each state therefore has its own healthcare delivery system in which both public and private (for profit as well as non-profit actors operate. While states are responsible for the functioning of their respective healthcare systems, certain responsibilities also fall on the federal (central) government, namely aspects of policy-making, planning, guiding, assisting, evaluating and coordinating the work of various provincial health authorities and providing funding to implement national programmes. The health subjects are divided into three groups: federal, concurrent and state. The state list is the responsibility of the state, including provision of medical care, preventive health services and pilgrimage within the state. At present there are 29 states in India, each state having its own health administration. At the village level, the elementary services are rendered by Village health guides, local dais, aganwadi workers, ASHA workers. The initiative to bring healthcare on the political agenda will have to be a multi-pronged one and fought on different levels. The idea here is not to develop a plan of action but to indicate the various steps and involvements that will be needed to build a consensus and struggle for right to healthcare.

10.8 KEY WORDS

Community Health Centres (CHCs): CHCs which constitute the secondary level of health care are designed to provide referral as well as specialist health care to the rural population. In addition to medical services, functions of CHCs also include making provisions for safe drinking water and basic sanitation, prevention and control of endemic diseases, collection of vital **statistics** of the area, health and nutrition, education.

Primary Health Care (PHC): Primary health care is essential health care based on practical, scientifically sound and socially acceptable methods and technology made universally accessible to individuals and families in the community through their participation. This includes physical, mental and social well-being and it is people-centered rather than disease-centered.

Accredited Social Health Activist (ASHA): An accredited social health activist is a community health worker instituted by the government of India's Ministry of Health and Family Welfare. ASHA create awareness on health and its social determinants and mobilize the community towards local health planning and increased utilization and accountability of the existing health services.

The National Rural Health Mission (NRHM): The National Rural Health Mission was launched by government of India to provide accessible, affordable and quality health care to the rural population, especially the vulnerable groups. The thrust of the mission is on establishing a fully functional, community-owned, decentralized health delivery system to ensure simultaneous action on a wide range of determinants of health such as water, sanitation, education, nutrition, social and gender equality.

10.9 FURTHER READINGS

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10.10 CHECK YOUR PROGRESS: POSSIBLE ANSWERS

Check Your Progress: 1

1. Refer to Section 10.2

Check Your Progress: 2

1. Refer to Section 10.4
2. Refer to Section 10.4.1

Check Your Progress: 3

1. Refer to Section 10.5
2. Refer to Section 10.5.1.3
3. Refer to Section 10.5

Check Your Progress: 4

1. Refer to Section 10.6